



THE GLOBAL AIDS MONITORING REPORT OF JAMAICA 2025

Reporting Period: January – December
2024



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TABLE OF CONTENTS

Table of Contents.....	1
List of Figures.....	4
List of Tables.....	4
List of Abbreviations and Acronyms.....	6
I: EXECUTIVE SUMMARY.....	10
II: preliminary section.....	14
Chapter 1: Introduction.....	14
Chapter 2: Background.....	17
1.1 Overview of Jamaica.....	17
1.2 Jamaica’s HIV Epidemic.....	17
1.3 The National HIV Response.....	19
II: Updates of Commitments and expanded targets to end aids.....	25
commitment 1: Combination HIV Prevention for All.....	25
1.1 Commitment 1 Indicator Performance Table.....	25
1.2 HIV prevention program for key and vulnerable populations.....	29
1.3 Risk behaviours, HIV prevention and HIV prevalence among Men who have sex with men and transgender women (MSM/TGW).....	31
1.4 Risk behaviour, HIV prevention and HIV prevalence among Female Sex Worker (FSW).....	32
1.5 HIV prevention for persons in prisons and other closed settings.....	33
1.6 Pre-Exposure Prophylaxis (PrEP).....	34
1.7 2024 Knowledge Attitudes, Behaviours and Practices (KABP) Survey.....	35
commitment 2: 95-95-95 for HIV Testing and Treatment.....	37
2.1 Commitment 2 Indicator Performance Table.....	37
2.2 HIV testing volume and positivity.....	39
2.3 HIV Treatment Cascade.....	40
2.3.1 Community Facilitators for improving the HIV Treatment Cascade.....	42
2.3.2 Integration of HIV treatment care and support services to improve linkage and retention in care.....	43
2.3.3 HIV Drug Resistance (DR) testing to improve viral suppression.....	43
2.4 Advanced HIV disease, late HIV diagnosis and AIDS mortality.....	44

2.5 Antiretroviral therapy coverage among PLHIV in key populations	44
Commitment 3: End Paediatric AIDS and Eliminate Vertical Transmission	45
3.1 Commitment 3: Indicator Performance	45
3.2 Elimination of Mother to Child Transmission (EMTCT)	46
3.3 Paediatric and Adolescent HIV/AIDS	47
Commitment 4: Gender Equality and Empowerment of Women and Girls	48
4.1 Commitment 4 Indicator Performance Table	48
4.2 Physical and/or sexual violence among key populations	49
4.3 Intimate Partner Violence	50
4.4 The Women's Centre of Jamaica Foundation (WCFJ)	51
4.5 Eve For Life	52
Commitment 5: Community Leadership	53
5.1 Participation of key populations	53
5.1.1 Mystery Shopper Assessments	53
5.1.2 Community Experience and Needs Assessment for LGBTQ+ persons in Jamaica	54
5.2 Participation on PLHIV and WLHIV	54
5.2.1 JN+ Community Scorecard	54
5.2.2 Jamaica Community of Positive Women (JCW+)	55
5.3 Participation of young people	56
Commitment 6: Realize Human Rights and Eliminate Stigma and Discrimination	57
6.1 Commitment 6 Indicator Performance Table	57
6.2 Eliminating Stigma and Discrimination in all settings.	59
6.3 Policy Initiatives to Address Human Rights and Eliminate Stigma and Discrimination ...	60
6.3.1 The National Workplace Policy on HIV and AIDS	60
6.3.2 The National Policy for the Management of HIV and AIDS in School	61
6.3.3 Data Protection Act	61
6.3.4 Policy Dialogue on Stigma and Discrimination and Human Rights	61
6.4 The Jamaica Anti-Discriminatory System for HIV (JADS)	62
6.5 Human Rights Training Impact Assessment	62
6.5 Legal Support Services and Access to Justice	63
6.6 Sensitization Sessions with key members of the population	63
Commitment 7: Universal Health Coverage	66

7.1 Commitment 7: Indicator Performance Table	66
7.2 Hepatitis in people living with HIV	67
7.3 Sexually transmitted infections	68
7.4 Coinfection of tuberculosis and HIV	68
7.5 Cervical Cancer.....	69
7.6 Differentiated service delivery (DSD) antiretroviral therapy models.....	69
7.7 Social Protection Services.....	70
Commitment 8: INVESTMENTS AND RESOURCES.....	71
8.1 Domestic public budget and expenditure for HIV	71
8.2 Resource Mobilization Efforts of Civil Society Organizations to Ensure Sustainability. ..	72

LIST OF FIGURES

Figure 1. Map of Jamaica depicting the geographical location of the island, the division by parish and by regional health authority.	17
Figure 2: Age at diagnosis and Sex Distribution of 41,518 reported cases of HIV in Jamaica....	18
Figure 3: The number of newly diagnosed HIV cases from 1982 - 2023 in Jamaica.....	18
Figure 4: Persons living with HIV per 100,000 population by parish for cases diagnosed 1982 – 2022.....	19
Figure 5: Public education campaign at major events to increase awareness of PrEP and provide on-demand PrEP and HIV testing on the spot.	35
Figure 6: Comparison of results from 2017 and 2024 of key GAM indicators from the Knowledge, Attitude, Behaviours and Practices population survey (KABP) among persons aged 15 - 49 years.....	36
Figure 7: Jamaica 95-95-95 HIV Treatment Cascade at the end of 2024. Data taken from the UNAIDS Spectrum Estimates Tool and the National Treatment Database.	40
Figure 8: Jamaica's continuum of care cascade for PLHIV at the end of 2024 as reported in the national treatment database.....	41
Figure 9: Implementation guide for the establishment of HIV services in primary health care in Jamaica.....	43
Figure 10: The proportion of participants in the MSM/TGW IBBS who reported ever experiencing life adverse events.	50
Figure 11: Photo of the participants in the Bureau of Gender Affairs Workshop in Mandeville..	51
Figure 12: The Women's Centre Foundation of Jamaica has supported 55,000 young women to successfully complete their education.	52
Figure 13: Social media posts of the "Mek Wi Par" campaign to address HIV-related stigma and discrimination in the family, household and community settings.	60
Figure 14: The organising entities capture a moment with the general manager-operations of Jamaica Teas Limited set to participate in the Beyond Labels: Building a Stigma Free Workplace Project.	64
Figure 15: Policy and Advocacy Officer JN+, Jaemar Johnson and Adherence Counselor Carlene Dawkins engages attendees in an FBO session at Kyle's Temple in St. Mary.	65

LIST OF TABLES

Table 1: National HIV policies and strategic plans in Jamaica	21
Table 2: Legislation that affects national HIV related policies in Jamaica	23
Table 3: Combination HIV Prevention for All Indicator Performance for 2024.....	25

Table 4 Estimated population size and reach targets and achievements for key populations FSW, MSM, TG, ARY, SAF and SAM in 2024	30
Table 5: Targeted behaviour addressed by the prevention programme and the number of interactions for the reporting period January - December 2024.	30
Table 6: Prevalence of HIV among men who have sex with men (MSM) and transgender women (TGW) in Jamaica from 1986 to 2024.	31
Table 7: Protective and risk factors associated with HIV infection among MSM and prevalence ratios with 95% confidence interval (95% C.I.)	32
Table 8: The prevalence of HIV among female sex workers in Jamaica from 1990 to 2024.	33
Table 9: Results on key GAM indicators from the Knowledge, Attitude, Behaviours and Practices population survey (KABP) of persons aged 15 - 49 years disaggregated by sex.....	36
Table 10: 95-95-95 for HIV testing and treatment indicator performance for 2024	37
Table 11: Testing volume and positivity of community-level testing.	39
Table 12: End paediatric AIDS and eliminate vertical transmission indicator performance for 2024.....	45
Table 13: Table showing Jamaica's performance against EMTCT targets for 2024.	46
Table 14: Gender Equality and empowerment of women and girls indicator performance for 2024.....	48
Table 15: Realize human rights and eliminate stigma and discrimination indicator performance for 2024.....	57
Table 16: The proportion of men and women aged 15 - 49 who express discriminatory attitudes towards persons living with HIV in Jamaica in 2024 by strongly disagree/disagree with the described scenarios.	60
Table 17: Universal health coverage indicator performance for 2024.	66
Table 18: Pathogens included in the STI 9 panel	68
Table 19: The planned and actual expenditure for the fiscal year April 1, 2023 - March 31, 2024, for the National HIV Programme.	71

LIST OF ABBREVIATIONS AND ACRONYMS

AHF	AIDS Healthcare Foundation
AIDS	Acquired Immunodeficiency Syndrome
ANC	Antenatal clinic
ARV	Antiretroviral
AYALHIV	Adolescent and Young Adults Living with HIV
BGA	Bureau of Gender Affairs
CBO	Community Based Organization
CF	Community Facilitators
CFA	Children First Agency
CLM	Community-Led Monitoring
CPFSA	Child Protection and Family Services Agency
CSO	Civil Society Organization
C-TECH	Caribbean Training and Education Centre for Health
CVCC	Caribbean Vulnerable Communities Coalition
DCS	Department of Correctional
DHIS-2	District Health Information Systems – 2
DSD	Differentiated Service Delivery ART Models
DVA	Domestic Violence Act
DVIC	Domestic Violence Intervention Centre
EAS	Enhanced Adherence Support
EEHR	Enabling Environment and Human Rights
EFAF	Equality for All Foundation
EMTCT	Elimination of Mother To Child Transmission
ETC	Entry to Care
FBO	Faith Based Organization
FHU	Family Health Unit
FSW	Female Sex Worker

GDP	Gross Domestic Product
Global Fund	Global Fund to Fight AIDS, Tuberculosis and Malaria
GOJ	Government of Jamaica
HATS	HIV/AIDS Tracking System
HCW	Health Care Worker
HIV	Human Immunodeficiency Virus
HPPB	Health Promotion and Protection Branch
HSTU	HIV/STI/Tb Unit
IBBS	Integrated Bio-Behavioural Study
IPV	Intimate Partner Violence
JADS	Jamaica Anti-Discriminatory System for HIV
JASL	Jamaica AIDS Support for Life
JCCM	Jamaica Country Coordinating Mechanism
JCW+	Jamaica Community of Positive Women
JFJ	Jamaicans For Justice
JFPA	Jamaica Family Planning Association
JN+	Jamaica Network of Seropositives
KABP	Knowledge, Attitudes, Behaviour and Practices
KVP	Key and Vulnerable Population
LPV/r	Lopinavir/ritonavir
LTFU	Loss-to-follow-up
M&E	Monitoring and Evaluation
MCGES	Ministry of Culture, Gender, Entertainment and Sports
MDA	Ministries, Department and Agencies
MLSS	Ministry of Labour and Social Security
MOEY	Ministry of Education and Youth
MoFPS	Ministry of Finance and Public Service
MOHW	Ministry of Health and Wellness

MOJ	Ministry of Justice
MSM	Men who have Sex with Men
NCDA	National Council on Drug Abuse
NERHA	Northeast Regional Health Authority
NFPB	National Family Planning Board
NGO	Non-Governmental Organisation
NHF	National Health Fund
NHP	National HIV/STI Programme
NPHL	National Public Health Laboratory
NSU	National Surveillance Unit
OPD	Office of the Public Defender
PAHO	Pan-American Health Organization
PATH	Programme for Advancement Through Health and Education
PEPFAR	President's Emergency Plan for AIDS Relief
PHDP	Positive Health Dignity and Prevention
PHDM	Positive Health & Dignity Movement
PIOJ	Planning Institute of Jamaica
PITC	Provider Initiated Testing and Counselling
PLHIV	Person Living with HIV
PLACE	Priority for Local AIDS Control Efforts
PrEP	Pre-Exposure Prophylaxis
PSOJ	Private Sector of Organisation
RGD	Registrar's General Department
RHA	Regional Health Authority
RHS	Reproductive Health Survey
S&D	Stigma & Discrimination
SDC	Social Development Corporation
SERHA	Southeast Regional Health Authority

SI	Strategic Information
STI	Sexually Transmitted Infection
SRH	Sexual and Reproductive Health
SRHA	Southern Regional Health Authority
TAJ	Tax Administration of Jamaica
Tb	Tuberculosis
TCS	Treatment, Care and Support
TG	Transgender Person
TLD	Tenofovir/Lamivudine/Dolutegravir
TLE	Tenofovir/Lamivudine/ Efavirenz
TPDCo	Tourism Product Development Company
TSIS	Treatment Sites Information System
TWG	Technical Working Group
UCSF	University of California, San Francisco
UIC	Unique Identifier Code
UNAIDS	Joint United Nations Programme on HIV/AIDS
UNDP	United Nations Development Programme
UNFPA	The United Nations Population Fund
UNICEF	United Nations Children Fund
USAID	United States Agency for International Development
VAWG	Violence Against Women and Girls
WHO	World Health Organisation
WHS	Women's Health Survey
WLHIV	Women Living with HIV
WRHA	Western Regional Health Authority
YA	Youth Ambassadors
YAM	Youth Advocacy Movement
ZLN	Zidovudine/Lamivudine/Nevirapine

I: EXECUTIVE SUMMARY

The Global AIDS Monitoring (GAM) report 2025, along with the National Commitments and Policy Instrument (NCPI), presents an analysis of Jamaica's progress in combatting the HIV/AIDS epidemic based on available data and current trends for the reporting period January – December 2024. While Jamaica has made significant strides in specific areas, particularly in achieving validation for elimination of mother to child transmission of HIV and syphilis (EMTCT) and integration of HIV prevention, treatment, care, support and enabling environment and human rights under a single entity within the Ministry of Health and Wellness, persistent challenges remain. The NCPI reflects a comprehensive policy framework for addressing HIV/AIDS. The efficiency of the policies is demonstrated in areas such as rapid treatment initiation, community-based service delivery, and the adoption of self-testing. However, inconsistencies in access to care, and the need for continued efforts to combat stigma and discrimination may affect the overall effectiveness of the HIV response. These challenges necessitate a renewed focus on targeted interventions, addressing social determinants of health and strengthening surveillance systems to achieve the UNAIDS 95-95-95 targets and ultimately, epidemic control.

Commitment 1: Combination HIV Prevention for All

The concept of combination HIV prevention for all aims to emphasise that prevention efforts must reach all populations and especially those facing the greatest risks and disparities. The response in the NCPI indicate that there is a strong foundation for combination prevention within the national HIV response. Jamaica has made strides in HIV prevention with 90% of persons estimated to be living with HIV in Jamaica aware of their status and a 112% increase in the number of persons initiating pre-exposure prophylaxis (PrEP). However, the 2024 data reveals a concerning trend of a widening gap between prevention efforts implemented and the needs of the most vulnerable populations. This was evidenced in the results from the integrated bio behavioural surveys (IBBS) within the men who have sex with men (MSM), transgender persons (TG) and female sex worker (FSW) populations and the completion of the knowledge, attitudes, behaviours and practices (KABP) survey within the general population. The IBBS studies showed that the overall prevalence of HIV remained relatively the same within the MSM (30% 2017 vs 27% 2024), TG (51% 2017 vs 54% 2024) and FSW (2% 2017 vs 3% 2024) populations but there is an increase in high risk sexual behaviour among these persons. This increase in high risk sexual behaviour is further underscored in the KABP survey where evidence shows declining HIV knowledge, increasing risk behaviours and persistent barriers such as stigma, discrimination and violence. It is envisioned that the integration of the prevention program into the HIV/STI/TB Unit in the Health Promotion and Protection Branch (HPPB) will allow for a more coordinated management of prevention efforts and a renewed commitment to data-driven strategies.

Commitment 2: 95-95-95 for HIV Testing and Treatment

The program demonstrates positive progress in achieving the 90-90-90 targets with high knowledge of status (90% of persons living with HIV aware of their status), improved antiretroviral therapy (ART) coverage (65% of persons aware of their status on ART), and increased viral suppression (80% of PLHIV on ART virally suppressed). This can be attributed to

the guidelines and policies that employ a wide range of HIV testing approaches and offers universal access to ART. However, the rise in advanced HIV disease at presentation suggests a need for strengthened efforts in early diagnosis and linkage to care. The program faces a "revolving door" phenomenon where individuals cycle in and out of care, despite efforts to return them. Younger individuals (≤ 29 years) and those in care for >3 years are particularly vulnerable to treatment interruptions, potentially due to treatment fatigue. In an effort to address these challenges, the Community Facilitators (CF) Programme, utilises peer-to-peer support and has demonstrated effectiveness in improving linkage, retention, and viral suppression through a personalized approach. Additionally, the Ministry of Health and Wellness has embarked on a process to integrate HIV services into primary health care to increase access points for all persons living with HIV.

Commitment 3: End Paediatric AIDS and Eliminate Vertical Transmission

In April 2024, Jamaica achieved a significant milestone by receiving World Health Organisation (WHO) validation for the elimination of mother-to-child transmission (EMTCT) of HIV and syphilis. Jamaica met the stringent targets of $<2\%$ HIV mother-to-child transmission rate, <0.3 annual rate of new HIV infections per 1000 live births, and <0.5 annual rate of congenital syphilis per 1000 live births from 2020-2023. This achievement places Jamaica among 19 countries globally with EMTCT certification. The country continues to sustain these gains in 2024 with 99.8% of pregnant women aware of their HIV status, 96.5% of those with HIV receiving antiretroviral therapy (ART) to prevent mother-to-child transmission (PMTCT) and robust syphilis screening and management. However, challenges remain in paediatric and adolescent HIV care with lower rates of viral suppression compared to adults. To address these challenges, the programme has implemented targeted approaches, including the deployment of community health aides (CHA) to improve follow-up and linkage to treatment.

Commitment 4: Gender Equality and Empowerment of Women and Girls

Gender equality is crucial in the fight against HIV/AIDS, as evidenced by the experiences of vulnerable populations in Jamaica. The Women's Centre of Jamaica Foundation (WCJF) continues to provide educational opportunities for adolescent mothers, helping them reintegrate into the formal education system after childbirth, reaching a milestone of assisting 55,000 young women in 2024. Additionally, the Bureau of Gender Affairs is actively working to address Intimate Partner Violence (IPV), a pervasive issue affecting 5.2% of ever-married or partnered women, with younger women (20-24 years) experiencing higher rates (9.5%). Government initiatives include direct support, skills training, collaboration with law enforcement, male engagement, and community training to address the systemic cultural factors contributing to IPV. However, key populations, including men who have sex with men (MSM), transgender women (TGW), and female sex workers, experience alarmingly high rates of physical and sexual violence, with TGW being disproportionately affected. A majority (66.7%) of female sex workers reported experiencing violence from sexual partners. These traumatic experiences lead to increased risk-taking behaviours, limited access to healthcare, and psychological distress, contributing to greater vulnerability to HIV. The data underscores the urgent need for comprehensive interventions and support systems to address the intersecting issues of gender inequality, violence, and HIV/AIDS.

in Jamaica. According to responses in the NCPI, there is at least one service delivery point that provides elements of comprehensive post-rape care as per WHO guidelines. However, the proportion of health facilities providing each element varies, which suggests inconsistencies in access to comprehensive care for survivors of sexual violence.

Commitment 5: Community Leadership

Key and vulnerable populations for HIV/AIDS actively participate in the national HIV response through a range of initiatives that drive meaningful change. The active participation of the LGBTQ+ is evidenced through two key initiatives: Mystery Shopper Assessments and a Community Experience and Needs Assessment. The Mystery Shopper exercise at public health care facilities revealed generally positive experiences regarding assistance and information sharing, but identified infrastructural and interpersonal challenges at specific locations. The Community Experience and Needs Assessment, underscored persistent challenges faced by LGBTQ+ individuals, including violence, discrimination, economic hardship, and mental health struggles. Additionally, people living with HIV (PLHIV) and women living with HIV (WLHIV) work very closely with the national HIV programme through advocacy and the provision of peer-to-peer mentorship programmes through the Jamaica Network of Seropositives (JN+) and the Jamaica Community of Positive Women (JCW+). This approach has been shown to be effective in improving treatment adherence, reducing viral load, increasing access to prevention and care services and advocating for improvements in the access to care for PLHIV. This collaborative and community-driven approach underscores the vital role of lived experience and peer support in achieving epidemic control and improving the lives of those affected by HIV.

Commitment 6: Realize Human Rights and Eliminate Stigma and Discrimination.

The recently conducted surveys have indicated that there is still a prevailing high proportion of persons with discriminatory attitudes towards persons living with HIV in Jamaica. The Jamaica Anti-Discriminatory System for HIV (JADS) reported 88 cases between January and December 2024, with the majority (82%) occurring in community settings, and harassment and verbal abuse comprising the majority (80%) of reported cases. JADS collaborates with various redress entities to address these cases. Additionally, the NCPI indicates that there are restrictive legal and policy frameworks that require attention and the need to invest in societal enablers. Policy initiatives conducted in 2024 to address these issues include the Ministry of Labour and Social Security's (MLSS) collaboration with the National Family Planning Board (NFPB) to revise the National Workplace Policy on HIV and AIDS, and the Ministry of Education, Skills, Youth and Information's partnership with the NFPB to revise the National Policy for the Management of HIV and AIDS in Schools, expanding it to include broader sexual and reproductive health concerns. Additionally, the Data Protection Act reinforces the right to privacy, crucial for people living with and affected by HIV. An impact assessment of human rights training delivered to public sector workers showed improvements in patient-centred care, stigma reduction, and ethical decision-making among healthcare workers. Sensitization sessions included healthcare worker training initiatives focused on issues affecting people living with HIV, training sessions with private sector professionals to promote inclusivity and create stigma-free workplaces, sessions with faith-based organizations and public duty bearers (police officers, justices of the peace and mediators) to

advance human rights advocacy, address HIV-related stigma and discrimination, and promote sexual health awareness.

Commitment 7: Universal Health Coverage and Integration

Universal health coverage strengthens health systems, reduces inequalities, and promotes human rights which are essential for achieving and sustaining HIV epidemic control. Jamaica is actively pursuing strategies and programmes that lead to the integration of HIV services however there is an urgent need to decentralize some treatment and care services within the national HIV programme. Additionally, there were efforts aimed at reducing the high rate of HIV co-infections with other communicable diseases while ensuring universal access to affordable prevention, diagnosis and treatment services. During the reporting cycle, there were limitations identified in the national monitoring and evaluation (M&E) systems to provide values for the indicators that measure this commitment. Nevertheless, Jamaica continues to demonstrate its commitment to universal health coverage and integration through various initiatives. There is continued emphasis on revisiting the approach to prevention, care and diagnosis of sexually transmitted infections (STIs) with the initiation of the activities required to revise the policy to include etiological diagnosis for STIs. Furthermore, the country has made significant progress in managing TB/HIV co-infection through effective screening and treatment programs, but maintaining consistent testing capacity is crucial to ensure continued success and prevent potential setbacks.

Commitment 8: Investments and Resources

The approved budget for the most recently completed fiscal year, April 1, 2023 – March 31, 2024 was approximately JMD 1.2 billion, with an overall expenditure rate of 69%. The GOJ contributed significantly, representing 75% of the approved budget, underscoring the nation's commitment to domestic funding and long-term program sustainability. Furthermore, in 2024, Jamaica finalized its application to the Global Fund for a new grant period starting in 2025. This will transition the program to activity-based contracts, aiming to enhance efficiency and accountability by linking funding directly to measurable results. The movement towards sustainability is also evident in the activities of civil service organisations in revising business plans and marketing strategies to enhance resource mobilisation and social enterprising efforts. This strategic shift reflects a dedication to optimizing resource allocation and strengthening the effectiveness of Jamaica's HIV initiatives.

II: PRELIMINARY SECTION

CHAPTER 1: INTRODUCTION

The Global AIDS Monitoring (GAM) framework, along with the National Commitments and Policy Instrument (NCPI), serves as a vital resource in assessing progress and identifying challenges in the global response to the HIV/AIDS epidemic. These reporting mechanisms are fundamental to effective monitoring and evaluation, enabling stakeholders to track advancements and make data-driven decisions. Regular surveillance of the HIV/AIDS epidemic is essential for guiding policy development, optimizing resource distribution, and shaping targeted interventions at both national and global levels. Through the systematic collection and analysis of key indicators, trends can be identified, intervention outcomes measured, and emerging challenges addressed. Standardized indicators play a crucial role in ensuring data comparability across different regions, facilitating an accurate assessment of progress toward international goals.

The United Nations General Assembly's Political Declaration on HIV/AIDS lays out a strategic roadmap for eliminating AIDS as a public health threat by 2030. This strategy is centred around three key priorities, each supported by specific focus areas. The first priority emphasizes equitable access to HIV services, including combination prevention approaches, universal HIV testing and treatment under the 95-95-95 framework, and the elimination of vertical transmission to end paediatric AIDS. The second priority focuses on dismantling barriers that hinder progress by advancing gender equality, empowering women and girls, promoting community leadership, and eradicating stigma and discrimination. The third priority underscores the importance of sustainable and well-funded HIV responses through integration with universal health coverage and strategic resource allocation. The annual GAM/NCPI report plays a pivotal role in tracking advancements toward these goals. Jamaica has aligned its national HIV/AIDS response with these global commitments, recognizing the critical role of monitoring, surveillance, and evaluation in achieving national targets.

To comprehensively assess progress on GAM/NCPI indicators and commitments, a consultative and participatory approach was adopted. This process involved gathering input from both internal and external stakeholders through direct engagement and discussion. The key elements of this approach included:

- Active stakeholder engagement
- Open dialogue and information exchange
- Qualitative data collection
- A flexible and iterative methodology
- Participatory decision-making
- Transparency and accountability

This inclusive approach ensured that insights from individuals and groups with relevant expertise were incorporated into the evaluation process. It also allowed for a deeper examination of key indicators and commitments, fostering consensus among stakeholders and enabling adaptive strategies in data collection based on evolving priorities.

Stakeholder	Organisation
1 Mrs. Rahanah Khan Francis	Chair, GAM Steering Committee, Ministry of Health and Wellness
2 Dr. Nicola Skyers	Ministry of Health and Wellness
3 Dr. Alisha Robb Allen	Ministry of Health and Wellness
4 Mr. Akeem Whitehead	Ministry of Health and Wellness
5 Dr. Yohann White	Ministry of Health and Wellness
6 Dr. Haneef Wilson	Ministry of Health and Wellness
7 Mrs. Amoy Douse-Mullings	Ministry of Health and Wellness
8 Ms. Selena Lewis	Ministry of Health and Wellness
9 Dr. Gail Evering Kerr	Ministry of Health and Wellness
10 Dr. Kereann Nelson	National Public Health Laboratory
11 Ms. Nardia McLaren	Bureau of Gender Affairs
12 Ms. Sherise Gayle	Attorney General Chambers
13 Inspector Raquel Longmore	Jamaica Constabulary Force
14 Mr. Conrad Saunders	Ministry of Labour and Social Security
15 Ms. Lilyclaire Bellamy	Jamaica Intellectual Property Office
16 Dr Peter Dawes	Department of Correctional Services
17 Ms. Nicola Cousins	Ministry of Health and Wellness
18 Ms. Shushan Stewart	Ministry of Health and Wellness
19 Mr. Colin Dosunmu	Ministry of Health and Wellness
20 Mrs. Kandasie Walton-Levermore	Jamaica AIDS Support for Life
21 Mr. Xavier Biggs	Jamaica AIDS Support for Life
22 Dr. Jennifer Tomlinson	Jamaica AIDS Support for Life
23 Mr. Kemar Bundy	C-TECH
24 Mr Jason Lawrence	C-TECH
25 Dr. Sharlene Jarrett	UCSF
26 Mr. Nickoy Wilson	Equality for All Foundation
27 Ms Jody-Ann Nugent	Jamaica Network of Seropositives

	Stakeholder	Organisation
28	Mr. Andwayne Davis	Jamaica Network of Seropositives
29	Ms. Olive Edwards	Jamaica Council of Positive Women
30	Ms. Novelette Howell	Women's Centre of Jamaica Foundation
31	Mrs. Beverly Martin-Berry	Women's Centre of Jamaica Foundation
32	Ms Abigail Lewin	Jamaica Family Planning Association
33	Ms Nahima Smith	Jamaica Family Planning Association
34	Dr. Lisa Pilgrim	Southeast Regional Health Authority
35	Dr. Serene Joseph	Pan-American Health Organization
36	Dr. Rodrigue Nze-Eyo	UNAIDS

CHAPTER 2: BACKGROUND

1.1 Overview of Jamaica

Jamaica, an island nation in the Caribbean, has a mixed public-private healthcare system, with the government playing a central role in providing healthcare services. The public healthcare system is primarily managed by the Ministry of Health & Wellness (MOHW) and operates through four Regional Health Authorities (RHAs): The South East, North East, Southern, and Western regions (**fig. 1**). These RHAs oversee hospitals, health centres, and community-based services to ensure accessible care across the island. Public healthcare facilities offer free services at the point of care, covering primary, secondary, and tertiary care, including maternal and child health, communicable and non-communicable disease management, and emergency services. The private healthcare sector supplements the public system, providing specialized and expedited care for those who can afford it. Jamaica's healthcare strategy emphasizes universal health coverage, health promotion, and strengthening its surveillance and response systems, particularly for infectious diseases such as HIV/AIDS and emerging health threats.

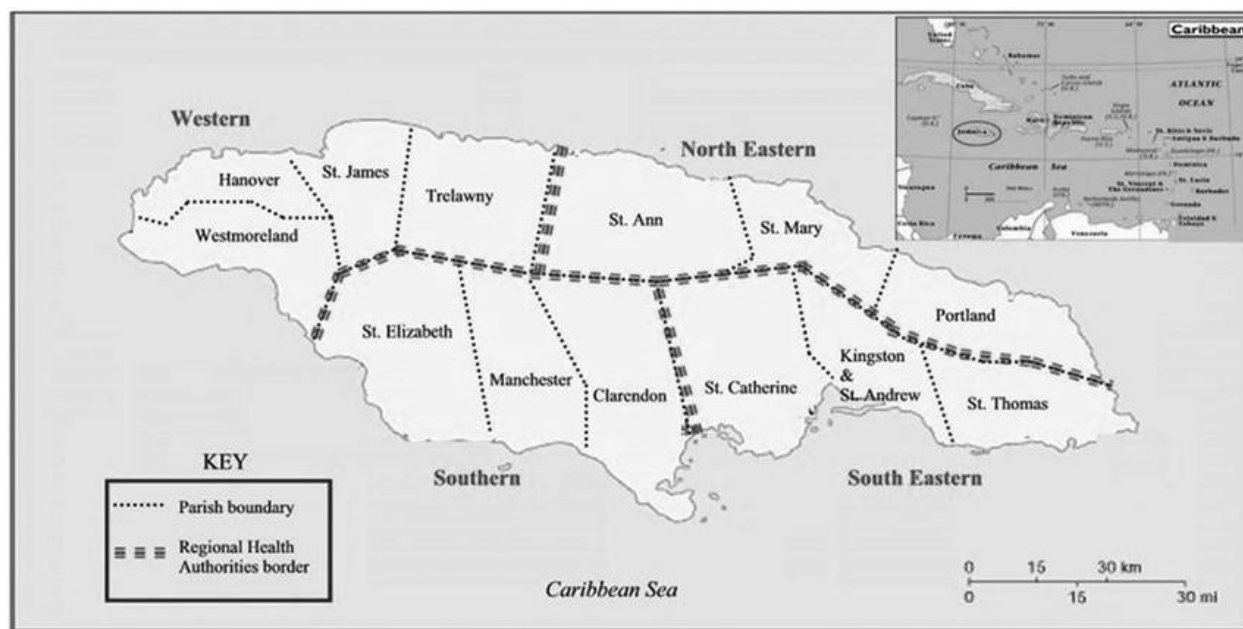


Figure 1. Map of Jamaica depicting the geographical location of the island, the division by parish and by regional health authority.

(Source: Paul Brown et al. 2013. *Leptospirosis and malaria as causes of febrile illness during a dengue epidemic in Jamaica. Pathogens and Global Health* 107 (6): 329-34.)

1.2 Jamaica's HIV Epidemic

The first reported case of HIV occurred in 1982, and since then a total of 41,518 cases have been reported to the National Surveillance Unit (NSU), with a male to female ratio of approximately 1:1 (**fig. 2**)¹. The number of reported cases drastically increased from 1 case in 1982 to 2,014 cases

¹ HIV Epidemiology Report. 2023. National Surveillance Unit, Ministry of Health and Wellness.

in 2006 when the highest number of cases were reported. Since then, there has been a 70% reduction in the number of cases reported between 2006 to 2023 (**fig. 3**).

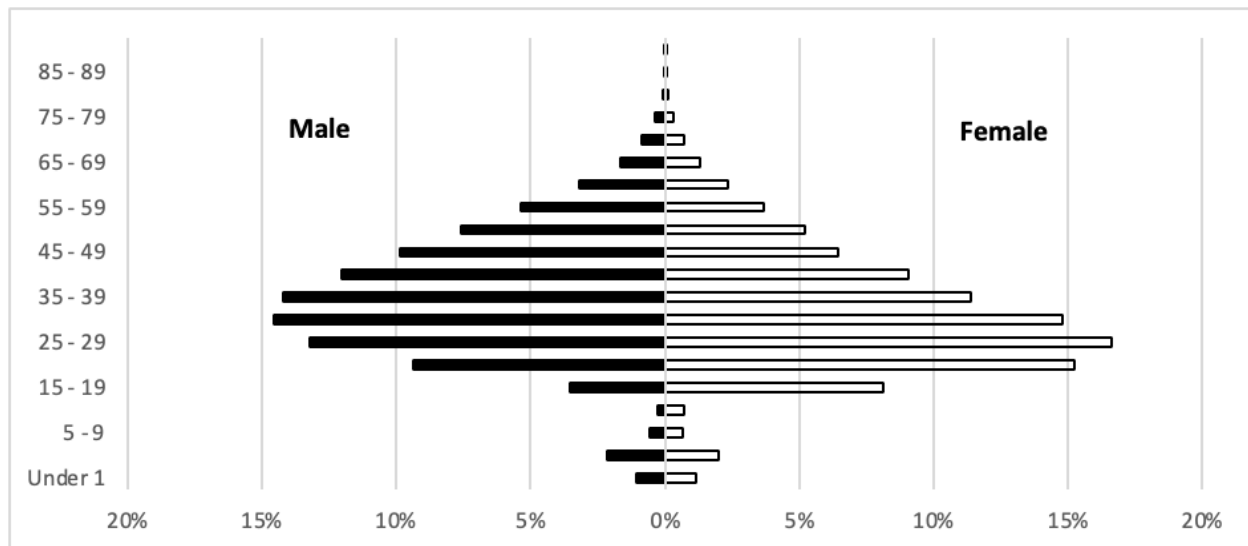


Figure 2: Age at diagnosis and Sex Distribution of 41,518 reported cases of HIV in Jamaica.

Source: HIV Epidemiological Report 2024.

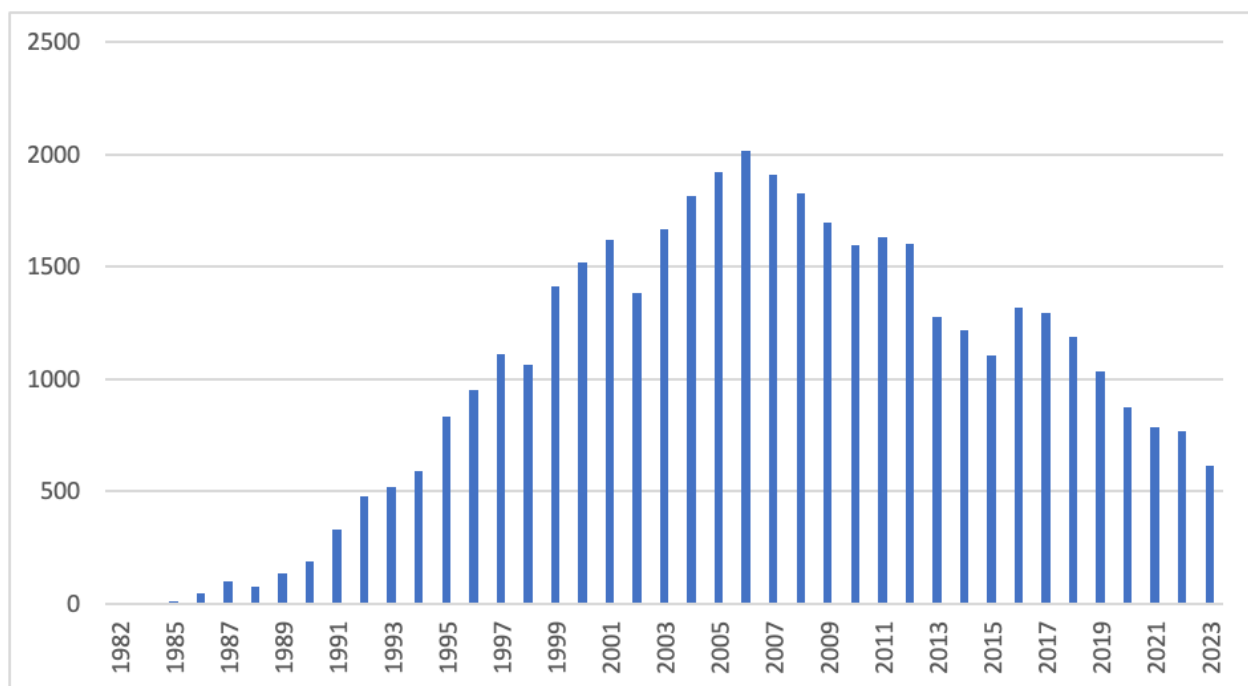


Figure 3: The number of newly diagnosed HIV cases from 1982 - 2023 in Jamaica.

Source HIV Epidemiological Report 2024.

Transmission of HIV in Jamaica is predominantly by sexual contact (71%) of all reported cases and as much as 95% of cases with a documented transmission category. The most identified risk

factors for HIV infection in Jamaica include history of sexually transmitted infections (29%), multiple sex partners (20%) and sex with a commercial sex worker (13%). These risk factors continue to predominate; of the 611 cases reported in 2023, 27% reported multiple sex partners, 24% reported history of sexually transmitted infections, and 10% reported sex with a commercial sex worker.²

A total of 15,035 deaths of PLHIV have been reported to the NSU. Currently there are 26,441 persons living with HIV who are aware of their status in Jamaica.³ The highest prevalence per 100,000 population is found in the parish of St. James followed by Kingston & St. Andrew and St. Ann. Notably the northern parishes that form the “tourism corridor” show a high prevalence per 100,000 (**fig. 4**). Additionally, a high prevalence is observed in key populations including MSM (30%), transgender women (51%) and female sex workers (2%).⁴

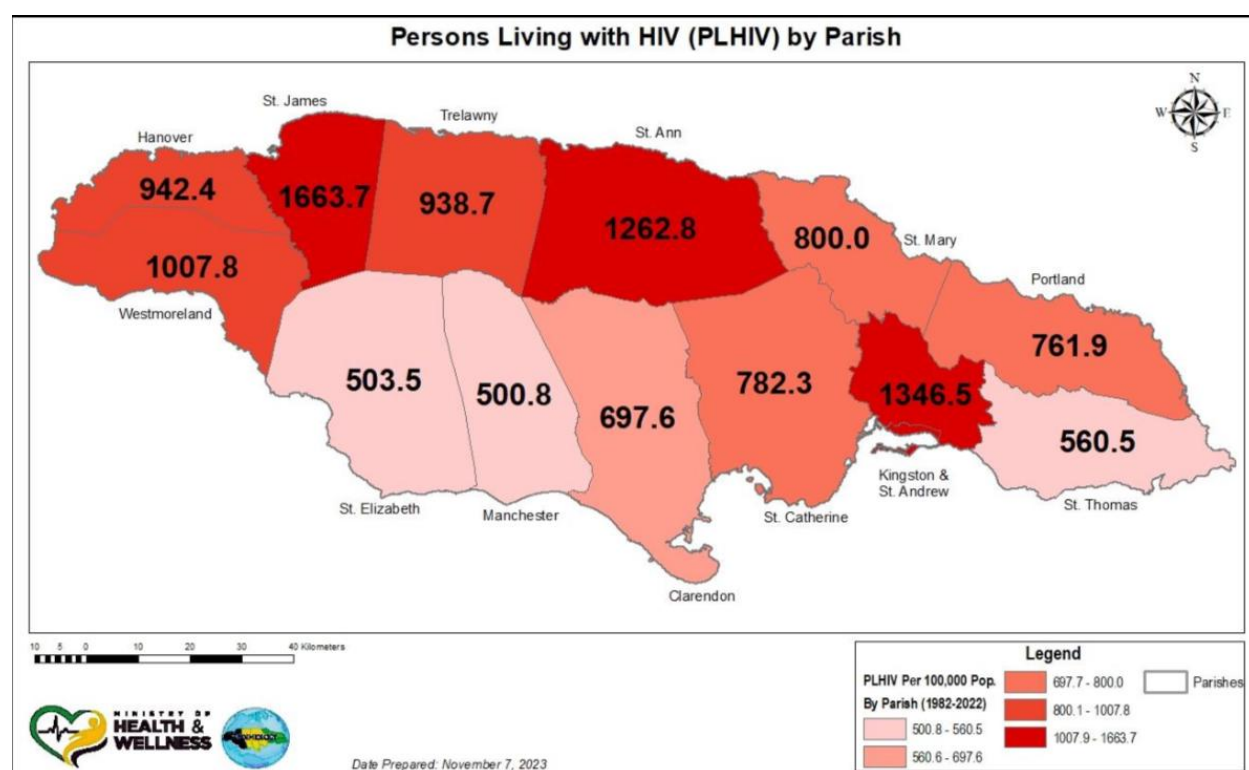


Figure 4: Persons living with HIV per 100,000 population by parish for cases diagnosed 1982 – 2022.

Source HIV Epidemiological Report 2023.

1.3 The National HIV Response

Since the first case of HIV/AIDS was reported in 1982, Jamaica has implemented a proactive strategy to manage the HIV epidemic, spearheaded by the Ministry of Health and Wellness (MOHW) through a coordinated multi-sectoral response. This includes involvement of

² HIV Epidemiology Report. 2024. National Surveillance Unit, Ministry of Health and Wellness.

³ HIV Epidemiology Report. 2024.

⁴ Ministry of Health and Wellness. The Global AIDS Monitoring Report of Jamaica. 2023

Government Ministries, Departments and Agencies (MDAs), civil society, private sector churches and persons living with HIV (PLHIV). Additionally, the response benefits from financial support and technical assistance from international, regional and bilateral partners, including the Global Fund to Fight AIDS, Tuberculosis and Malaria (GFATM) and the President's Emergency Plan for AIDS Relief/Centres for Disease Control and Prevention (PEPFAR/CDC) with additional support from various international development partners including the United Nations (UN) entities (UNAIDS, UNICEF, UNDP and UNFPA)⁵.

Currently the coordination and implementation of the national HIV response is managed by the HIV/STI/Tb Unit (HSTU) in the Health Promotion and Protection Branch (HPPB) of the Ministry of Health and Wellness (MOHW). During 2024, the MOHW successfully integrated the leadership of the HIV programme into a single entity, with the HSTU now monitoring all arms of the response to include prevention, treatment, care and support and enabling environment and human rights (EEHR) after dissolution of the National Family Planning Board (NFPB). This strategy aims to reduce fragmentation and streamline health promotion efforts to improve efficiencies in the utilization of resources and enhance sustainability of the national HIV response.

During this period, key steps were taken to decentralize all laboratory services, integration of the procurement of ARVs and HIV products and services into the National Health Fund (NHF), integration of HIV treatment services in all health centres offering curative services akin to other chronic diseases and upscaling the public-private partnership model for HIV treatment and care services. The national HIV response is on target to achieve these key strategic initiatives which aim to achieve universal access to care for PLHIV and contribute to the MOHW's goal for Primary Health Care Reform. This vision will see a strengthening of delivery of population based and individual care services with an integrated service delivery model for improved transitions through the continuum of care, increased access points with the inclusion of private healthcare providers, upgraded health facilities and multidisciplinary teams for comprehensive patient-centred services and improved health information systems.

Also integral to the efficient governance of the national HIV response is the re-establishment of the National AIDS Committee (NAC) to ensure active participation of all sectors and stakeholders within the response. The NAC was established in 1988, however the committee has been inactive within the last six years with the Country Coordinating Mechanism (CCM) stepping in to take on a coordinating role in the interim. A National AIDS Coordinating Body Steering Committee was established and provided recommendations for the implementation of a national coordinating structure. Central to the re-establishment of the NAC is to ensure there is alignment with the requirements of the current evolved HIV response and have commitment from the Government in its implementation.

An area of significant challenge in the country has been the reduction of stigma and discrimination, even though a number of policies have been developed over the years to create an enabling environment for the HIV response including The Public Health Act (1985, amended in 2014), The

⁵ Ministry of Health and Wellness. The Global AIDS Monitoring Report of Jamaica. 2023. For reporting period January – December 2022.

National HIV/AIDS Policy (2005 revised 2017), The National Policy for HIV/AIDS Management in Schools (2004), The Plan of Action for Orphans and Other Children Made Vulnerable by HIV/AIDS (2003), The Draft Strategic Framework for HIV/AIDS for incarcerated populations in Jamaica (2009) and the National HIV/AIDS Workplace Policy (2011) (**table 1**).⁶ However, there is some inconsistency between government and HIV-related priorities which is evidenced by specific legislation that continues to affect the national HIV response including the Offences against the Person Act (1864, amended in 2014), the Towns and Communities Act Section 3r (1843, amended in 1995) and Law Reform (Age at Majority) Act (1979)(**table 2**).⁷ In an attempt to address these consistencies, a Sexual and Reproductive Health (SRH) policy was developed to include access to care for minors and key populations and address the legislative barriers that impact the sexual and reproductive health needs of the population. This policy is now under review by the MOHW in preparation for Cabinet Submission.

Within the national HIV response framework, it is recognized that private practitioners, civil society organisations (CSOs) and community led organizations (CLOs) are able to provide treatment, care and support to a subset of PLHIV and other members of key and vulnerable populations. This is made possible through the standardization of care through implementation of approved policies and procedures between the national HIV response and private practitioners and CSOs. However, challenges related to the institutionalised role of CSOs in the national response, capacity building of CSO partners and sustainable funding sources may significantly derail the achievements of the partnerships with CSOs. It is within this context that the national HIV response aims to establish social contracting between the MOHW and non-government organizations (NGOs).

Table 1: National HIV policies and strategic plans in Jamaica⁸

Policy/Strategic Plan	Description
The Plan of Action for Orphans and Other Children Made Vulnerable by HIV/AIDS (2003)	Proposes social, educational, organizational and administrative approaches for assisting children affected by HIV/AIDS.
National Policy for HIV/AIDS Management in Schools (2004)	Outlines anti-discrimination guidelines, purports support for students affected by HIV and AIDS and promotes the information and education of all stakeholders.
The National HIV/AIDS Policy (2005 revised in 2017)	Establishes broad-based approaches to HIV/AIDS and its effect on the entire population. Focuses on HIV prevention and treatment and upholds the rights of persons living with HIV.

⁶ Althea Bailey et al. 2020. HIV policy in Jamaica: good form and substance but inadequate application and accountability. *Global Journal of Medicine and Public Health* 9 (5).

⁷ Althea Bailey et al 2020.

⁸ Althea Bailey et al 2020.

Policy/Strategic Plan	Description
The Draft Strategic Framework for HIV/AIDS for incarcerated populations in Jamaica (2009)	Proposes guidelines for HIV prevention testing and treatment among persons detained and in correctional institutions as well as their partners.
The National Workplace Policy on HIV/AIDS (2011)	Addresses the rights of workers infected and/or affected by HIV and reduction of stigma and discrimination at the workplace through education and training
National Policy for Gender Equality (2011)	Sets a multi-sectoral framework for achieving gender equality and advancing the rights of women and girls. Identifies modalities for securing gender equality in policies and programmes across social sectors.
National Policy Reintegration of School-Age Mothers into the Formal School System (2013)	Sets a multi-sectoral framework for achieving gender equality and advancing the rights of women and girls. Identifies modalities for securing gender equality in policies and programmes across social sectors.
Social Protection Strategy (2014)	Creates a framework for social protection, including the equitable delivery of social services using the life-cycle approach.
Health and Family Life Education Policy (2015)	Defines the policy scope and plan for implementing the Ministry of Education's Health and Family Life curriculum which includes modules on sex and sexuality education including HIV.
National Strategic Plan for the Elimination of Gender Based Violence 2017 – 2027	Consolidates national goals and targets for eliminating gender based violence, mitigating its effects and improving coordinated responses and service delivery.
Strategic Plan for Positive Health Dignity and Prevention (2018)	Creates a framework for the greater involvement of people living with HIV in the national response.
GOJ Service Excellence Policy (2022)	Clarifies the GOJ's commitment to providing the highest quality of service to all its customers and defines the mechanisms for institutionalising service excellence across the public sector.
National Community-Led Monitoring Strategy for Jamaica 2023 – 2026	Creates a framework for community-led monitoring in the national HIV response
Draft 2023 – 2030 National Strategic Plan for HIV	Creates a framework for the national HIV response

Table amended from: Althea Bailey et al. 2020. *HIV policy in Jamaica: good form and substance but inadequate application and accountability*. *Global Journal of Medicine and Public Health*, 9 (5).

Table 2: Legislation that affects national HIV related policies in Jamaica⁹

Law	Description
Offences against the Person Act (1864, amended in 2014)	Lists offences against the person including homicide, assault, suppression of brothels, child stealing, bigamy, abortions, infanticide, unnatural offences (specifies anal sex).
Towns and Communities Act Section 3r (1843, amended in 1995)	Makes provision for the maintenance of good order in towns and communities.
Dangerous Drug Act (1948, amended in 2013)	Regulates the importation, exportation, manufacture, sale and use of opium and other dangerous drugs
Law Reform (Age of Majority) Act (1979)	Sets at 16 the age at which a child can access medical services without the need for parental consent.
The Public Health Act (1985 sec. 2(1), 14(1))	Makes provisions for promoting public health and preventing the spread of communicable and epidemic diseases. Categorizes HIV as a notifiable disease and authorizes medical professionals to apply measures to prevent and provide treatment. Secures the removal of legislative barriers for PLHIV to access education and employment.
Data Protection Act (2020)	Seeks to safeguard privacy and personal information of Jamaicans. This supports the guidelines on confidentiality that are states in the Clinical Management of HIV Disease guidelines and the guidelines for the prevention of vertical transmission of HIV and Syphilis.
Domestic Violence Act (2004 amended 2023)	Provides remedies for domestic violence for the protection of the survivor/victim through speedy effective relief and for matters connected there with and incidental there to.
Child Care and Protection Act (2004)	Establishes a mandatory reporting framework for all forms of abuse, neglect or harm impacting a child. Health care workers are among those required by law to provide reports if they reasonably suspect that a child, has been, is being, or is likely to be, abandoned, neglected or physically or sexually ill-treated or otherwise in need of care and protection.

⁹ Althea Bailey et al 2020.

Law	Description
Sexual Offences Act (2009)	Comprehensively addresses and criminalizes a wide range of sexual offenses, including rape and sexual assault, while establishing a sex offender registry.
Disabilities Act (2014)	Prohibits discrimination against persons with disabilities and promotes their equality of access to health services. Recognizes in law the right of persons with disabilities to have access to the same range and standard of affordable health services as provided to other persons, including sexual and reproductive health services and population-based health programmes.
The Occupational Health and Safety Bill (2017)	Provides a draft framework to secure the safety and health of workers and bans discrimination based on infection or disease but not yet in force.
Sexual Harassment Act (2022)	Contains provisions for dealing with sexual harassment in the workplace, schools, correctional institutions, places of safety, nursing homes, medical and psychiatric facilities, among other places.

Table amended from: Althea Bailey et al. 2020. HIV policy in Jamaica: good form and substance but inadequate application and accountability. *Global Journal of Medicine and Public Health*, 9 (5).

II: UPDATES OF COMMITMENTS AND EXPANDED TARGETS TO END AIDS

COMMITMENT 1: COMBINATION HIV PREVENTION FOR ALL

1.1 Commitment 1 Indicator Performance Table

Table 3: Combination HIV Prevention for All Indicator Performance for 2024

Indicator	Definition	2024 Performance	Previous Data (date reported)	Comments
1.1 HIV incidence	Number of people newly infected with HIV in the reporting period per 1000 uninfected population	0.42	0.46	SPECTRUM
1.2 Estimates of the size of key populations	Number of people engaging in the specific behaviour that put the given population at risk for HIV transmission	-	-	-
A	Sex Workers	13,820 (2024)	19,689 (2012)	IBBS
B	Gay men and other men who have sex with men	27,000	20,923 (WHO)	IBBS
D	Transgender people	-	2,781 (2017)	IBBS
E	People in prisons and other closed settings	3571 (2024)	3,527 (2023)	Programme Data
1.3 HIV prevalence among key populations	Percentage of specific key population living with HIV	-	-	-
A	Sex Workers	2.5% 25/1011 (2024)	2% 16/805 (2017)	IBBS
B	Gay men and other men who have sex with men	27.2% 125/459 (2024)	30% 164/537 (2017)	IBBS

Indicator	Definition	2024 Performance	Previous Data (date reported)	Comments
1.3 cont'd HIV prevalence among key populations	Percentage of specific key population living with HIV			
D	Transgender people	53.7% 22/41 (2024)	51% 52/102 (2017)	IBBS
E	People in prisons and other closed settings	2.0% 70/3571 (2024)	1.9% 67/3527 (2023)	Programme data
1.4 HIV testing and status awareness among key populations	Percentage of people from key populations who report having tested negative for HIV in the past 12 months, or who know that they are living with HIV			
A	Sex Workers	88% 897/1020 (2024)	79% 954/1207 (2017)	IBBS
B	Gay men and other men who have sex with men	-	86% 462/537 (2017)	IBBS
D	Transgender people	100% 41/41 (2024)	81% 83/102 (2017)	IBBS
1.5 Condom use among key populations				
A	Percentage of sex worker reporting using a condom with their most recent client	89.9% 917/1020 (2024)	67% 805/1207 (2017)	I0.4BBS

Indicator	Definition	2024 Performance	Previous Data (date reported)	Comments
B	Percentage of men reporting using a condom the last time they had anal sex with a male partner	67.5% 308/457 (2024)	71% 381/537 (2017)	IBBS
D	Percentage of transgender people reporting using a condom during their most recent sexual intercourse or anal sex	63.4% 26/41 (2024)	61% 62/102 (2017)	IBBS
1.6 Coverage of HIV prevention programmes among key populations	Coverage of HIV prevention programmes: percentages of people in a key population reporting having received a combined set of HIV prevention interventions			
A	Sex Workers	49.1% 8319/16940 (Programme) 63.6% 649/1020 (IBBS 2024)	43% 8378/19689 (2023)	Programme Data IBBS 2024
B	Gay men and other men who have sex with men	21.3% 5760/27000 (Programme) 27% 125/459 (IBBS 2024)	33% 6932/20923 (Programme)	Programme Data IBBS 2024
D	Transgender people	12% 344/2781 (Programme) 54% 22/41 (IBBS 2024)	11% 314/2781 (2023)	Programme Data IBBS 2024

Indicator	Definition	2024 Performance	Previous Data (date reported)	Comments
1.7 HIV prevention programmes in prisons and other closed settings	HIV prevention and treatment programmes offered to people in prisons and other closed settings while detained			
	Receiving antiretroviral therapy	70	67	Programme Data
	Tested for HIV	571	421	Programme Data
	Living with HIV	70	67	Programme Data
	With Hepatitis C	0	0	Programme Data
	Coinfected with HIV and hepatitis C	0	0	Programme Data
	With Tb or coinfectd with HIV and Tb	2	0	Programme Data
1.11 People who received pre-exposure prophylaxis	Number of people who received pre-exposure prophylaxis (PrEP) at least once during the reporting period	602	284	Programme Data
1.14 Condom use at last high-risk sex	The percent of respondents who say they used a condom the last time they had sex with a non-marital, non-cohabiting partner, of those who have had sex with such a partner in the last twelve months	60.0% 165/275 Male 53.1% 34/64 Female (2024)	90.9% Male62.3% Female (2017)	KAPB Survey (2024)
1.15 Annual number of condoms distributed	Number of condoms distributed during the past 12 months			
	Male Condoms	1,330,420 pieces	4,933,405 pieces	Programme Data

Indicator	Definition	2024 Performance	Previous Data (date reported)	Comments
1.16 Young people: knowledge about HIV prevention	Percentage of women and men 15-24 years old who correctly identify both ways of preventing the sexual transmission of HIV and reject major misconceptions about HIV transmission	27.3% 272/998 (2024)	33% (2017)	KAPB Survey (2024)

Notes:

1. Indicators 1.2C, 1.3C, 1.4C, 1.5C, 1.6C, 1.8, 1.9, 1.10 are not relevant to Jamaica because of the low prevalence of persons who inject drugs.
2. Indicator 1.7 condoms are not distributed to people in prisons or other confined settings due to legislation that exists.

1.2 HIV prevention program for key and vulnerable populations

The prevention response of the programme was tasked with reaching 54% of the estimated FSW population, 48% of the estimated MSM population, and 28% of the estimated TG population. The national programme was not able to attain the 2024 targets as shown in **table 4**. The challenges faced by the prevention programme in reporting period 2023 continued into 2024. However, the integration of the management of the prevention programme and staff into the Health Promotion and Protection Branch (HPPB) was completed in August 2024. This marked the completion of a 6-year long process to rationalise the public bodies that offer services through the public sector transformation programme and the passing of the National Family Planning Board (repeal) Bill in 2023. While this move of integration is a welcomed step, the process resulted in loss of productivity within the prevention team as there is an increased need for on boarding and training on how to effectively deliver the HIV prevention programme.

Based on the epidemiological situation of HIV within the country adolescents and at-risk youth have been identified as a vulnerable population within society at an increased risk of acquiring HIV. Notable challenges exist in reaching this population, especially with the prevention team being legally prohibited from offering the comprehensive package of services to adolescents under 16 years of age. Similarly, sexually active males and females are a vulnerable population with sexually active females being easier to reach with prevention services than sexually active males, a trend which has remained consistent.

Table 4 Estimated population size and reach targets and achievements for key populations FSW, MSM, TG, ARY, SAF and SAM in 2024

Key Population	Population Size	2024 Reach Targets	2024 Reach Achievements	Percent Achievement
FSW	19,689	10,824	8,319	77%
MSM	20,923	10,044	5,760	57%
TG	2,718	768	3,344	45%
ARY*	-	24,390	21,203	87%
SAF^	-	96,297	42,006	44%
SAM`	-	90,932	28,953	32%

*Adolescents and At-Risk Youth (ARY),^Sexually Active Female (SAF), `Sexually Active Male (SAM)

In an effort to tailor the prevention package to the population at increased risk for acquiring HIV, the prevention programme identifies targeted behaviours to address during the reporting period. From January – December 2024 the targeted behaviours and the number of interactions are described in **table 5**. Furthermore, targeted mass-media campaigns developed by civil society organisations, such as the “Love It Right” campaign, specifically ensures the prevention communication and behaviour change interventions are designed for impact within the key populations such as high-risk FSW, TG and MSM.

Table 5: Targeted behaviour addressed by the prevention programme and the number of interactions for the reporting period January - December 2024.

Targeted Behaviour Addressed	Number of Interactions
Partner Reduction	31,535
Family Planning	14,378
Substance Abuse	11,924
Abstinence	9,857
Violence Reduction	9,347
Anal/Genital Care	8,723

1.3 Risk behaviours, HIV prevention and HIV prevalence among Men who have sex with men and transgender women (MSM/TGW)¹⁰

A survey was conducted in 2024 to evaluate behaviours, social experiences and HIV and syphilis prevalence in men who have sex with men (MSM) (n = 459) and transgender women (TGW) (n = 41). HIV prevalence in MSM was 27.2% (95% C.I. 22.9% - 31.6%) and in TGW was 53.7% (95% C.I. 21.3% - 69.6%). There was no significant change in the prevalence of HIV within the population as shown in **table 6**.

Table 6: Prevalence of HIV among men who have sex with men (MSM) and transgender women (TGW) in Jamaica from 1986 to 2024.

Year	Number surveyed	HIV prevalence
1986	125	9.6%
1995	201	32.3%
2007	207	32.9%
2011	449	31.4%
2017	537 MSM 102 TGW	29.6% 51.0%
2024	459 MSM 41 TGW	27.2% 53.7%

There was a high prevalence of sexual risk behaviours among MSM respondents with 50.3% reporting an STI in the past 12 months, 59.8% with 3 or more partners in the past 12 months and 35.5% reporting receiving money for sex. There was a low risk perception among MSM with 38.6% of respondents indicating that they had little or no chance of getting HIV, while 22.0% of TGW indicating a moderate chance of getting HIV.

The use of condoms and PrEP significantly reduces the risk of transmission of HIV; however, only 25.5% MSM and 14.6% TGW reported having ever taken PrEP even though 87.0% MSM and 92.7% TGW had heard about PrEP. Furthermore, 58.8% and 48.2% of MSM and TGW respectively reported using a condom the last time they had sex, which represents a significant reduction in condom use within the population from 71% MSM and 61% TGW in 2017.

Various factors were identified that were associated with a high prevalence (risk factors) or low prevalence (protective factors) of HIV. The prevalence ratios reported in **table 7** indicates how

¹⁰ MSM and TGW Survey 2024 in Jamaica. December 2024. Ministry of Health and Wellness and Jamaica AIDS Support for Life.

much more or less likely that a person will contract HIV. As indicated taking PrEP has a powerful protective effect and highlights the need for scaling up PrEP access, awareness and adherence within the MSM population. While, history of any STI and an STI in past 12 months are strong risk factors for HIV infection and suggests a need to improve STI screening, diagnosis and treatment and continued integration of STI services with HIV testing and prevention.

Table 7: Protective and risk factors associated with HIV infection among MSM and prevalence ratios with 95% confidence interval (95% C.I.)¹¹

Factor	Prevalence ratio	95% C.I.
Take PrEP	0.37	0.20 – 0.68
Completed High School or Tertiary education	0.45	0.33 – 0.65
16+ age at first sex	0.65	0.44 – 0.96
Ever told STI	4.5	2.8 – 7.3
STI past 12 months	1.9	1.3 – 2.6
Ever raped	1.63	1.18 – 2.23
Ever in jail or prison	1.5	1.1 – 2.1
Ever homeless	1.7	1.2 – 2.3
Ever violently attacked by partner	1.5	1.2 – 1.9
Sexually abused as a child	1.4	1.0 – 2.0
Ever received money for sex	1.8	1.2 – 2.6
Received money for sex in past 6 months	1.5	1.1 – 2.1

1.4 Risk behaviour, HIV prevention and HIV prevalence among Female Sex Worker (FSW)

A survey was conducted in 2024 to evaluate behaviours, social experiences and HIV and syphilis prevalence in female sex workers (n = 1660). The HIV prevalence in the current FSW population was 2.5%. As shown in **table 8** HIV prevalence among Jamaican female sex workers saw a dramatic decrease from a high of 12% in 1990 to a low of 2% in 2017. However, the recent report of an increase to 2.5% in 2024 highlights the need for continued monitoring and programme adaptation.

Condom use is high (92.6%) among current female sex workers who engage in sexual activity with a new paying client but only 36% reported using a condom with their main partner. Almost all current sex workers (93%) reported easy access to condoms with 66.6% participating in a condom demonstration activity in the past 6 months.

¹¹ MSM and TGW Survey 2024 in Jamaica. December 2024.

Table 8: The prevalence of HIV among female sex workers in Jamaica from 1990 to 2024.

Year	HIV prevalence
1990	12% ¹²
1997	9% ¹²
2005	9% ¹²
2008	4.9% ¹²
2011	5.4% ¹²
2014	3.2% ¹³
2017	2% ¹⁴
2024	2.5% ¹⁵

As indicated previously, PrEP use significantly reduces the risk of transmission of HIV but awareness and use of PrEP is very low among female sex workers. Only 26.5% were aware of PrEP and only 4.1% had ever used PrEP. Similarly, there is very low knowledge on the use of PEP after a single high risk event among current female sex workers (22.6%) and only 4.8% had ever used PEP.

The data presented in the female sex worker survey highlights gaps and challenges in current prevention and treatment strategies among this key population.

1.5 HIV prevention for persons in prisons and other closed settings

This aspect of the national HIV response is managed by the Department of Correctional Services (DCS) under the auspices of the Ministry of National Security (MNS). HIV testing is conducted by informed consent and is systematically offered to all new admissions to the adult correctional institutions and upon request by physicians. In 2024, there were a total of 433 persons who were tested for HIV upon new admissions. At the end of 2024, there were 70 PLHIV in prisons and all are on treatment. Prevention services such as condom distribution and PrEP are not offered in prisons and represents a contentious issue nationally due to the interpretation of various legislations specifically affecting gay men and other men who have sex with men.

For persons in other closed settings, including ‘lock-up’ (detention), prevention services are also provided. These services have oversight from the regional level, however the activities are not standardized across all regions. In the WRHA, prevention teams visit these ‘lock-ups’ and offer HIV testing, however because of the non-standardization of this process, the monitoring and evaluation surrounding this activity is limited. In addition to prevention, all persons in lock-ups who have disclosed to the authorities or who have disclosed to family members are able to receive their medication while in lock-up. However, these persons receive their medication from their enrolled clinics and therefore the information is not tracked by the department of correctional services. In some instances, where the authorities are made aware, they accompany the detainees

¹² Figueroa JP et. al. 2020. The HIV epidemic in Jamaica: a need to strengthen the National HIV programme. Rev Panam Salud Publica. 2020 Nov 20;44: e157. doi: 10.26633/RPSP.2020.157. PMID: 33245295; PMCID: PMC7679044.

¹³ Weir SS et. al. 2018. Reaching key populations through key venues: Insights from the Jamaica HIV Prevention Programme. PLoS One. 2018 Nov 26;13(11) 206962. doi: 10.1371/journal.pone.0206962. PMID: 30475802; PMCID: PMC6261031.

¹⁴ Skyers N et. al. The 4th (2017) Generation Surveillance of Commercial Sex Workers, Female Patrons and Workers of Sites where persons meet sex partners or participants in sexual activity in Jamaica. (Unpublished Report)

¹⁵ Ministry of Health and Wellness. The 5th (2024) Generation Surveillance of Commercial Sex Workers, Female Patrons and Workers of Sites where persons meet sex partners or participants in sexual activity in Jamaica. (unpublished report)

to their regularly scheduled clinical appointments. Also, for persons who have disclosed to family members, the family informs the health facility and collects medication on behalf of their family member.

1.6 Pre-Exposure Prophylaxis (PrEP)

While there are multiple PrEP products that have been authorized for use, the country currently uses tenofovir 300mg/emtricitabine 300mg (TDF/FTC) for Daily PrEP or On Demand PrEP. The target groups for PrEP include:

- Sexually active MSM
- Heterosexually active men and women with concurrent or high number of sex partners
- Transgender women
- Heterosexual men and women whose partners are known to be HIV+ (sero-different couples)
- Sexually active adolescent girls, boys, young women and men
- Sex workers
- Individuals with recurrent STI and repeated use of post-exposure prophylaxis (PEP)

The prevention programme within the national HIV response continued to increase knowledge and awareness surrounding the use of PrEP in high risk groups. They conducted targeted interventions by attending major events and activities where persons gather to enhance visibility and to provide On Demand PrEP at these events (**fig. 5**). Additionally, the robust education and communication campaign that began in 2023 was continued throughout the reporting period of January to December 2024.

Anecdotal evidence provided at the Annual Treatment Care and Support forum in 2024 has indicated that the low uptake of PrEP within the country may be related to specific structural and procedural barriers in how PrEP was administered specifically, the initiation being conducted at designated HIV treatment sites and delays in waiting for laboratory results prior to initiation. In order to address these challenges, the Centre for HIV/AIDS Research and Education Services (CHARES) clinic introduced “PrEP on the Go” initiative. Key elements of this initiative include :

- Same day eligibility screen and PrEP initiation
- Provision of 14-day starter pack of medication
- Comprehensive medical interviews and sample collection
- Integration of psychosocial support
- Implementation of an electronic appointment system.

The introduction of this delivery modality increased the number of persons who started PrEP by 137% (from 25 to 92) and was able to retain 99% of those who started PrEP. This activity highlighted how impactful same-day PrEP initiation is within the population and the need to utilise this modality routinely in an effort to provide PrEP to those who it would most benefit.

At the end of 2024, there were 602 persons initiated on PrEP, which represents a 111% increase from the number initiated in 2023. Also, the Jamaica AIDS Support for Life (JASL) PrEP clinic reported that 63% of all persons who initiate PrEP have returned for at least one follow-up visit.



Figure 5: Public education campaign at major events to increase awareness of PrEP and provide on-demand PrEP and HIV testing on the spot.

1.7 2024 Knowledge Attitudes, Behaviours and Practices (KABP) Survey

The knowledge attitudes, behaviours and practices (KABP) survey on HIV/AIDS was completed in 2024 and provides valuable insights to tailor effective interventions. By assessing current knowledge levels, identifying misconceptions and understanding prevalent attitudes and behaviours, the national HIV response can develop targeted campaigns that address knowledge gaps, challenge stigma and promote safer practices.

Key risk factors are investigated during the KABP survey including age at sexual debut, multiple sex partners in the past 12 months and condom usage (**table 9**). Sex before age 15 increases vulnerability to HIV due to a combination of biological factors and social factors. Based on the preliminary results from the survey approximately 1/3 of males had sexual debut before age 15 years. In addition to a high proportion of males having an earlier sexual debut, approximately 30% of males report multiple sexual partners in the past 12 months. Having multiple sex partners

increasing risk of HIV exposure by increasing likelihood of encountering a partner living with HIV and making condom use inconsistent. Condom use significantly reduces the risk of HIV transmission, however the KABP 2024 survey indicated that a majority of the population are not using condoms with only 41.8% of males and 32.9% of females reporting using a condom at last sex. Taken together these findings highlight an urgent need for targeted interventions to promote safer sexual practices.

Table 9: Results on key GAM indicators from the Knowledge, Attitude, Behaviours and Practices population survey (KABP) of persons aged 15 - 49 years disaggregated by sex.

Indicator	Male (15-49)	Female (15-49)
Sexual debut < 15 years	27.0%	6.3%
Multiple Sex partners in past 12 months	27.5%	6.4%
Condom use during last 10 sexual intercourse	25.1%	19.4%
Condom use last sex with regular or non-regular partner	41.8%	32.9%

In comparing key indicators from the KABP survey in 2024 and 2017, there is a concerning decline in accurate HIV knowledge (40% in 2017 vs 30% in 2024) and proportion of persons who received an HIV test in the last 12 months (50% in 2017 to 35% in 2024). This data combined with other findings revealed a troubling trend where persons are more likely to be exposed to HIV and less equipped to protect themselves and know their status. Also, there was an increase in the number of persons reporting multiple partners in the last 12 months (17% in 2017 to 23% in 2024) with only a marginal increase in the proportion of these respondent that indicated using a condom the last time they had sex (56% in 2017 to 59% in 2024). Notably, there was a dramatic decrease in the number of persons reporting transactional sex in the past 12 months, which requires further investigation (**fig. 6**).

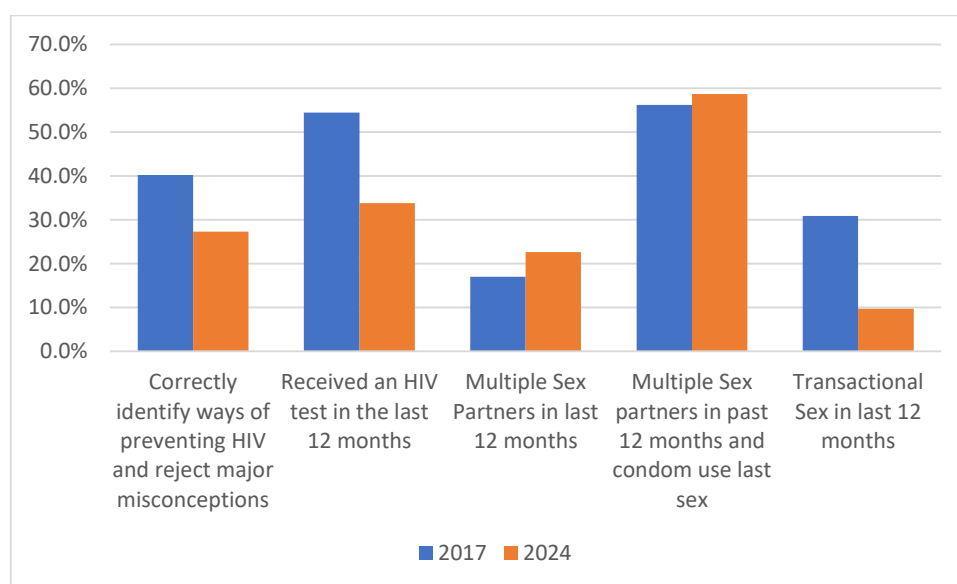


Figure 6: Comparison of results from 2017 and 2024 of key GAM indicators from the Knowledge, Attitude, Behaviours and Practices population survey (KABP) among persons aged 15 - 49 years.

COMMITMENT 2: 95-95-95 FOR HIV TESTING AND TREATMENT

2.1 Commitment 2 Indicator Performance Table

Table 10: 95-95-95 for HIV testing and treatment indicator performance for 2024

Indicator	Definition	2024 Performance	Previous Data (date reported)	Comments
2.1 People living with HIV who know their HIV status	Percentage of people living with HIV who know their HIV status at the end of the reporting period	95% 26,500/28,000	90%23,500/26,000	SPECTRUM and National Surveillance Unit
2.2 People living with HIV on antiretroviral therapy	Percentage and number of adults and children on antiretroviral therapy among all adults and children living with HIV at the end of the reporting period	54% 15,215/28,000 (2024)	57% 14,755/26,000 (2023)	Treatment Database and SPECTRUM
2.3 People living with HIV who have suppressed viral loads	Percentage and number of adults and children living with HIV who have suppressed viral loads at the end of the reporting period	44% 12,192/28,000 (2024)	45% 11,673/26,000 (2023)	Treatment Database and SPECTRUM
2.4 Advanced HIV disease and late HIV diagnosis	Percentage and number of adults and children with CD4 cell count <200 cells/mm ³ (or <15%) at initial diagnosis or initiation/re-initiation of antiretroviral therapy during the reporting period	50% 194/388 (2024)	43% 370/851 (2023)	Programme Data 2024 – numerator is the number who initiate/reinitiate and have a CD4 test done within one (1) month
2.5 HIV testing volume and positivity	The number of HIV tests conducted (testing volume) and the percentage of HIV-positive			

Indicator	Definition	2024 Performance	Previous Data (date reported)	Comments
	results returned to people (positivity) in the calendar year.			
	Community-level testing	0.4% 638/176,781 (2024)	0.5% 464/90027 (2023)	Programme Data
2.5 cont'd HIV testing volume and positivity	Facility level testing	1.1% 3,137/281,273 (2024)	1.1% 2,537/221,696 (2023)	Programme Data
	Number of self-test kits distributed	15,198	12,439	Programme Data
2.6 Antiretroviral therapy coverage among people living with HIV in key populations	Percentage of people living with HIV in a key population receiving antiretroviral therapy in the past 12 months			
A	Female sex workers	68% 17/25 (2024)	-	IBBS
B	Gay men and other men who have sex with men	84.9% 110/124 (2024)	51% 84/164 (2017)	IBBS
D	Transgender people	86.4% 19/22 (2024)	27% 14/52 (2017)	IBBS
E	People in prisons and other closed settings	100% 70/70 (2024)	100% 67/67 (2023)	Programme Data
2.7 AIDS mortality	Total number of people who have died from AIDS-related causes per 100 000 population	31.99	32.15	SPECTRUM

Note:

1. Indicator 2.6C is not relevant to Jamaica because of the low prevalence of persons who inject drugs.
2. Indicator 2.8 data not available because national reporting does not capture the information.

2.2 HIV testing volume and positivity

The country is estimated to have achieved the 1st 90 target of having 90% of persons living with HIV being aware of their status, which is a testament to the national testing strategies. In total, there were 311,723 tests conducted at the facility and community-levels with 3,001 positive results giving a positivity yield of 1.0%. The relatively low positivity yield of 1.0% suggests that the testing strategies are reaching a broad population, but ongoing surveillance and targeted interventions remain critical to further reduce transmission.

HIV self-test kits have been introduced to expand the national testing strategies in 2021. In 2024, a total of 15,198 self-test kits were distributed from the national stocks. Data on the distribution of 5,965 self-test kits was reported to the national HIV response and of these results from 531 tests was recorded. A total of 16 tests were reactive giving a positivity yield of 3.0%. The data highlights the fact that while self-test kits offer convenience and privacy, there is significant underreporting of results and an inability to link tests to the individual.

Facility-level testing includes all HIV tests conducted at the hospital and at the health centres. This includes all ante-natal care (ANC) testing, testing for all admissions to the hospital and testing for all visits to the health centre in an effort to make HIV testing a routine part of healthcare services offered in public health facilities. In 2024, 51% of all admissions to hospitals and 14% of all health centre visits were tested for HIV. There was 1.1% positivity rate from all facility level tests conducted.

The populations targeted with community-level testing include female sex workers, men who have sex with men (MSM), transgender persons (TG), sexually active males and females and at-risk youth. A total of 176,781 tests were conducted with an overall positivity rate of 0.4%.

However significantly higher positivity rates were seen with 3.6% in the MSM population and 7.1% in the TG population (**table 12**). The data coincides with the higher burden of HIV in the MSM and TG populations in Jamaica.

Table 11: Testing volume and positivity of community-level testing.

Key population	2024 Reach Achievement	Testing target (90% of those reached)	Testing Achievement (%)	Positive Results (Positivity Rate %)
FSW	8,319	7,488	8,017 (96%)	66 (0.8%)
MSM	5,760	5,184	5,630 (98%)	202 (3.6%)
TG	344	310	326 (95%)	23 (7.1%)
Sexually active females (SAF 25 – 49)	42,006	37,806	39,652 (94%)	127 (0.3%)
Sexually active males (SAM 25 – 49)	28,953	25,734	25,409 (88%)	75 (0.3%)

At risk youth (16 – 24)	21,203	19,083	19,184 (90%)	45 (0.2%)
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2.3 HIV Treatment Cascade

As of the end of 2024, an estimated 26,000 persons were living with HIV in Jamaica. Approximately 90% (23,500) of PLHIV know their status, 65% (15,215) of those who know their status are on ART and 80% (12,192) of those on ART are virally suppressed.

Review of the national 95-95-95 cascade reveals a mixed picture. While 90% of people living with HIV (PLHIV) are aware of their status, indicating strong testing efforts, only 65% of those diagnosed are on antiretroviral therapy (ART) and 80% of those on ART are virally suppressed (**fig. 7**). The most significant gap is in the second 95, indicating a major challenge in linking people who know their status to ART. This suggests potential barriers to accessing or adhering to treatment. While the third 95 is also not met, the gap is smaller than the ART enrolment gap. This suggests that once people are on ART, a reasonable proportion achieve viral suppression. However, there is room for improvement in adherence and treatment effectiveness.

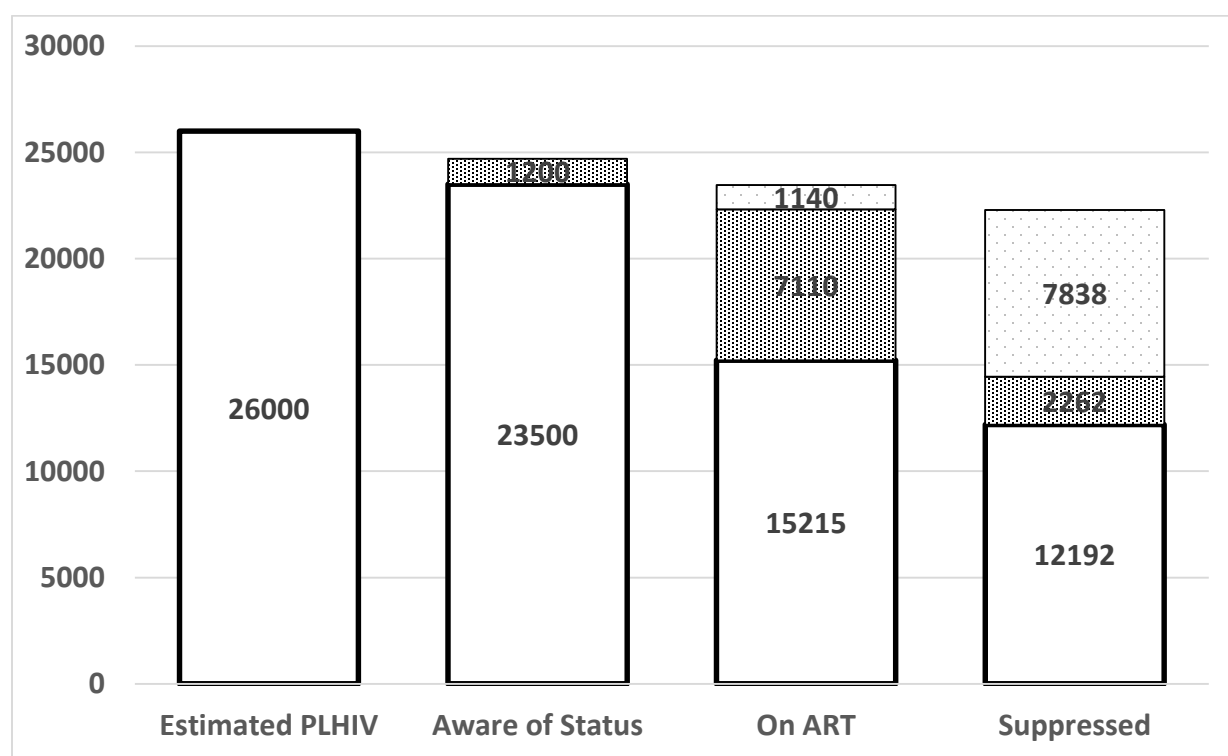


Figure 7: Jamaica 95-95-95 HIV Treatment Cascade at the end of 2024. Data taken from the UNAIDS Spectrum Estimates Tool and the National Treatment Database.

Following our analysis of the 95-95-95 targets, we now delve into the continuum of care cascade to pinpoint specific areas needing attention (**fig. 8**). Of the 19,419 individuals linked to care, 83% (16,184) were retained, indicating strong initial engagement. However, while the majority of those retained were on antiretroviral therapy (ARV) – 94% of the retained individuals, representing 78%

of all those linked to care – we observe a further decline. Specifically, 87% of those on ARV received a valid viral load test, which accounts for 68% of the total individuals linked to care. Ultimately, 64% of the initial group achieved viral suppression. This cascade highlights two key areas with significant barriers: retention in care and viral load uptake. The largest gap is in retention, with 3,235 individuals lost to follow-up (LTFU), defined as missing a scheduled appointment by more than 90 days. This necessitates strategies to improve appointment adherence. Additionally, 1,939 individuals on ARV did not receive a valid viral load test within the past 12 months, suggesting potential barriers to timely testing. Addressing these gaps in retention and viral load testing is crucial for optimizing the continuum of care and improving viral suppression rates.

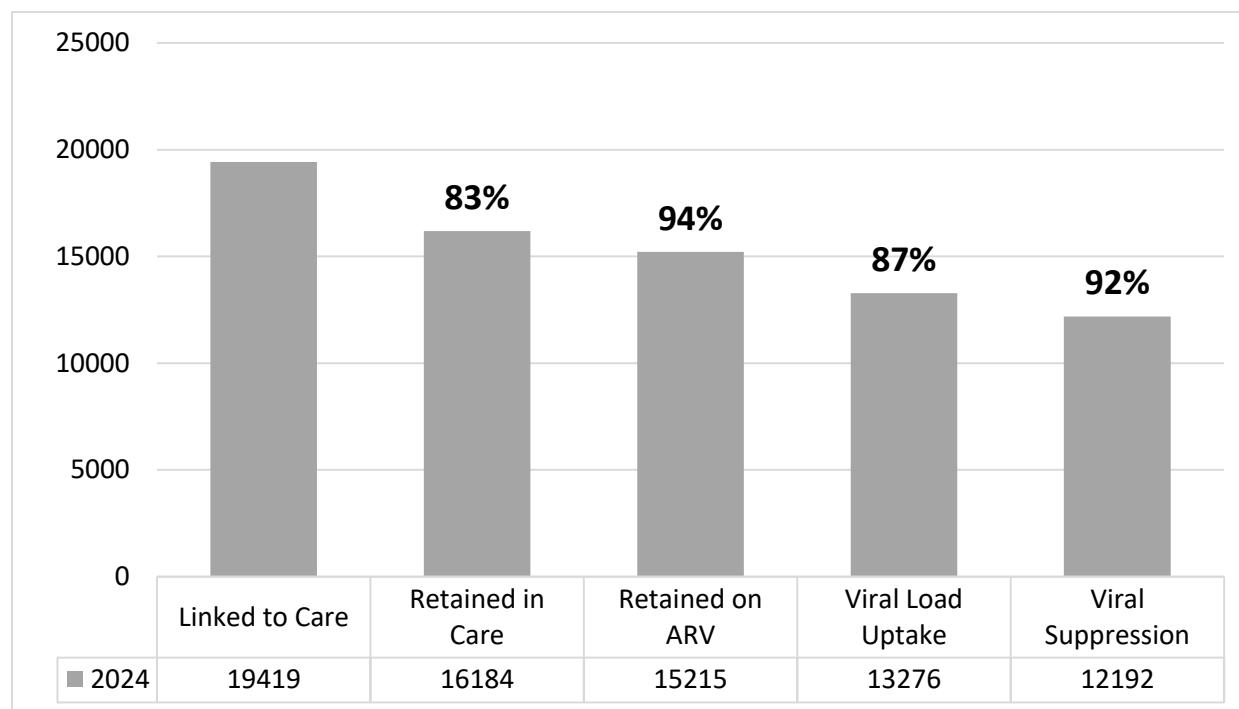


Figure 8: Jamaica's continuum of care cascade for PLHIV at the end of 2024 as reported in the national treatment database.

One of the main findings of investigations into retention among PLHIV in Jamaica is the “revolving door” phenomenon. PLHIV enter a cycle of recurrent treatment interruptions, where they return to care for a short period of time and then subsequently miss their next appointment. This was highlighted at the treatment site which boasts the highest retention rate of 90% over the past 3-years, but there seems to have been no improvement above 90%.

A study to investigate the factors associated with recurrent treatment interruptions identified that persons initiating care aged 29 years and younger were more likely to have frequent interruptions in care. Additionally, the persons more likely to cycle through periods of treatment interruption were in care for greater than 3 years or had shorter times between appointments, highlighting the possible impact of treatment fatigue, fuelling disengagement in HIV treatment and care services. Taken together, this information suggests that comprehensive strategies are needed that do not focus solely on returning lost patients but also incorporate activities that foster long-term

adherence, preventing recurrent interruptions. Therefore, programmes such as the community facilitators peer-to-peer approach as described below that facilitates a personalized, patient centred approach will continue to be prioritized in the treatment, care and support services offered to PLHIV. Additionally, a move towards more flexible appointment scheduling with integration of HIV services in primary care will be implemented and continued public-private partnerships.

2.3.1 Community Facilitators for improving the HIV Treatment Cascade

The community facilitator deployment programme through the Jamaican Network of Seropositive (JN+) plays an integral role in linkage, retention, suppression and return to care initiative within the national HIV response. The programme achieves this through direct peer-to-peer support of PLHIV. In an assessment of the programme for the period 2018 – 2022 the majority of beneficiaries were knowledgeable about HIV treatment (medication/retention), self-care, condom negotiation, sexual risk management and stigma and discrimination, highlighting the success of the programme to promote empowerment and leadership among PLHIV. From 2019 – 2022 1096 PLHIV were supported of which 787 (72%) were retained in care and maintained on ART and 519 (66% of those retained) achieved viral suppression. In 2023, 602 PLHIV received support and 79% (478) received a viral load test within 12 months with 359 (75% of those with a viral load test) achieving viral suppression. By August 2024, 976 PLHIV received support of which 799 were eligible for viral load testing and of those 534 (66% of those eligible) received a viral load test and 372 (70% of those with a viral load test) achieved suppression.

The successes of the CF programme is achieved through the personalized, creative approaches employed by community facilitators (CF's). The disclosure of their own HIV status with their peers establishes trust and confidentiality among them and has proven to be effective in engaging PLHIV who may be hesitant to adhere to treatment and clinic appointments. However, it was highlighted that even with this specific targeted approach by the CF, it is the individual's inherent motivation to stay in care and take medication that will ultimately lead to viral suppression.

2.3.2 Integration of HIV treatment care and support services to improve linkage and retention in care

For 2024, 26 health centres were assessed to determine eligibility to offer integrated HIV treatment and care services. Of these, 5 were identified as eligible but required improvements. Several key challenges were identified that hindered the effective integration of HIV services into primary healthcare. These included infrastructure deficiencies, resource and guidance gaps and human resource limitations. These findings indicate that successful integration of HIV services into primary healthcare in Jamaica requires:

- Investment in infrastructure, including network connectivity and facility upgrades.
- Development and distribution of current, MOHW-compliant guidelines and manuals.
- Comprehensive staff training to enhance clinical skills and improve the patient experience.

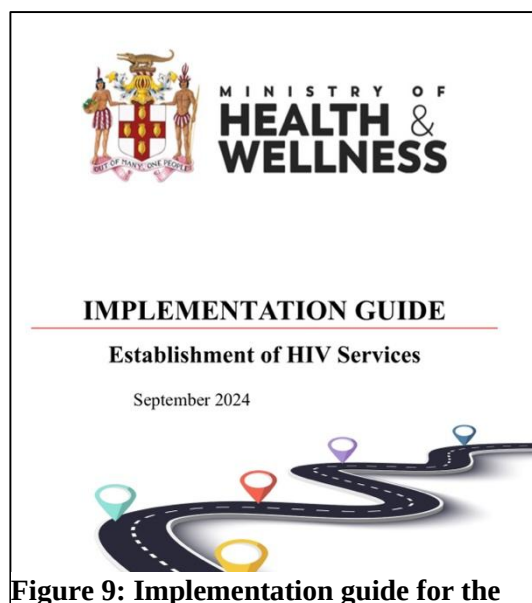


Figure 9: Implementation guide for the establishment of HIV services in primary health care in Jamaica

Following this assessment, the integration of HIV services was piloted at 5 health centres and an implementation guide was developed by the HSTU Ministry of Health and Wellness to guide the continued rollout by the regional health authorities. This implementation guide provides the steps, resources and activities to be complemented pre-implementation and during implementation (**fig. 9**).

2.3.3 HIV Drug Resistance (DR) testing to improve viral suppression

Drug resistance testing is an integral part in achieving and maintaining viral suppression within the national HIV programme. By identifying resistance mutations, clinicians can optimize treatment regimens, ensuring the continued effectiveness of antiretroviral therapy (ART). Moreover, routine resistance testing provides valuable surveillance data, allowing the program to monitor resistance trends, inform treatment guidelines, and adapt strategies to emerging challenges. After a pause in drug resistance testing following the COVID pandemic, testing resumed in 2024 at the National Public Health Laboratory. This testing included samples that had been collected from 2019 – 2024 allowing for the programme to catch up on potential resistance that developed during the period of reduced monitoring. A total of 64 samples from PLHIV with treatment failure was tested in 2024. Results indicate that most samples have high-level resistance to nucleoside reverse transcriptase inhibitors (NRTI; $\approx 50\%$) and/or non-nucleoside reverse transcriptase inhibitors (NNRTI; $> 90\%$). Very few samples had any mutation that conferred resistance to integrase inhibitors (INSTI; $< 15\%$) and protease inhibitors (PI; $< 10\%$).

It is also noted that the Protocol for the Management of HIV Drug Resistance was also completed in 2024 in an effort to enhance the capacity of providers of HIV care to provide evidence based information on HIV drug resistance according to international standards. Taken together these initiatives aim to ensure the long-term sustainability of HIV treatment, care and support programmes in the country.

2.4 Advanced HIV disease, late HIV diagnosis and AIDS mortality

An assessment of the number of persons with late stage diagnosis of HIV revealed that from 1990 – 2016 the majority of persons were diagnosed with either AIDS or advanced HIV. However, from 2017 – 2020 this rate dropped to below 45%, but this still represents greater than one-third of new diagnoses. Analysis of national level data revealed that the majority of late presenters are male and aged over 50 years. Further investigation also revealed that the majority of persons (50%) who presented late during the period 2017 to 2022 had died by the time of the study in 2024. While there has been a significant reduction in late-stage HIV diagnoses since 2017, it remains a critical concern, especially among older persons and males. The high mortality rate among those diagnosed late emphasizes the urgent need for targeted interventions to improve early detection and linkage to care.

2.5 Antiretroviral therapy coverage among PLHIV in key populations

Understanding the HIV treatment cascade within each key population is crucial for identifying gaps and disparities in access to and engagement with care. The cascade tracks the proportion of people living with HIV who are aware of their status, linked to care, on antiretroviral therapy (ART), and virally suppressed. By examining the cascade for specific key populations, such as men who have sex with men, transgender individuals or sex workers, we can pinpoint where interventions are most needed. This targeted approach allows for the development and implementation of tailored strategies to improve HIV testing rates, linkage to care, treatment adherence, and ultimately, viral suppression, leading to better health outcomes and reduced transmission within each population.

The 2024 IBBS studies have indicated a high proportion of MSM (84.9%) and TG (86.4%) who know their status are on ART. However, the data from FSW indicate a concerning trend of only 68% of respondents who know their status are on ART.

Jamaica AIDS Support for Life (JASL) continues to be one of the premiere treatment locations for PLHIV from the key populations. Of the 45 clients newly linked to care at JASL in 2024, 87% (39/45) identified as members of key populations, of which, almost all (97% 38/39) identified as MSM. Furthermore, of all persons registered at JASL treatment sites across the country 76% are virally suppressed

COMMITMENT 3: END PAEDIATRIC AIDS AND ELIMINATE VERTICAL TRANSMISSION

3.1 Commitment 3: Indicator Performance

Table 12: End paediatric AIDS and eliminate vertical transmission indicator performance for 2024.

Indicator	Definition	2024 Performance	Previous Data (2023)	Comments
3.1 HIV testing in Pregnant Women	Percentage of pregnant women with known HIV status	99.8% 24351/24410	99% 27977/28381	Programmatic Data
3.2 Early Infant Diagnosis	Percentage of infants born to women living with HIV receiving a virological test for HIV within two months of birth	96.5% 277/287	96% 350/365	Programmatic Data
3.3 Vertical transmission of HIV	Estimated percentage of children newly infected with HIV in the past 12 months due to vertical transmission	0.7% 2/287	0.5% 2/365	Programmatic Data
3.4 Preventing vertical transmission of HIV	Percentage of pregnant women living with HIV who received antiretroviral medicine to reduce the risk of vertical transmission of HIV	96.5% 279/289	98% 359/365	Programmatic Data
3.5 Syphilis among Pregnant women	Percentage of women accessing antenatal care services who were:			Programmatic Data
	Tested for syphilis	97.7% 23863/24410	98% 27716/28381	
	Tested positive	0.8% 199/23863	0.8% 220/27716	
	Treated appropriately	93% 185/199	96% 211/220	

Indicator	Definition	2024 Performance	Previous Data (2023)	Comments
3.6 Congenital Syphilis rate (live births and stillbirth)	Percentage of reported congenital syphilis cases (live births and stillbirth)	0.1% 14/24410	0.03% 9/27988	Programmatic Data

Note:

1. Indicator 3.7 data not available because national reporting does not capture this information

3.2 Elimination of Mother to Child Transmission (EMTCT)

In April 2024, Jamaica received validation from the World Health Organization (WHO) for the elimination of mother-to-child transmission (EMTCT) of HIV and syphilis. The achievement follows the implementation of interventions to strengthen the primary prevention and treatment services for HIV and syphilis within the Maternal and Child Health (MCH) services in Jamaica, which is ongoing. The programme attained the targets of < 2% HIV mother to child transmission rate, < 0.3 annual rate of new HIV infections per 1000 live births and < 0.5 annual rate of congenital syphilis per 1000 live births from 2020 – 2023. To date, 19 countries have received certification globally, with Jamaica being the largest English-speaking Caribbean State attaining the milestone.

The results from the reporting period January 1 – December 31, 2024, showed that the country continues to meet most targets for EMTCT outcome indicators and speaks to the efforts at sustaining the gains of the PMTCT program (**table 13**). The national HIV response continued to engage the field routinely to ensure that syphilis reactive pregnant women are appropriately treated to prevent vertical transmission.

Table 13: Jamaica's performance against EMTCT targets for 2024.

Indicator	Target	Numerator	Denominator	Percentage
Percentage of pregnant women who were tested for HIV and received their results during pregnancy, during labour and delivery, and during the postpartum period (<72 hours), including those with previously known positive HIV status	> 95%	24,351	24,410	99.8%
Percentage of pregnant women tested for Syphilis during pregnancy	> 95%	23,863	24,410	98%
Percentage of HIV-positive pregnant women who received anti-retroviral to reduce the risk of MTCT	> 95%	279	289	97%
Percentage of Syphilis-seropositive pregnant women who are appropriately treated	> 95%	185	199	93%

National Programmatic Data 2024.

3.3 Paediatric and Adolescent HIV/AIDS

Since 2023, there has been a concerted effort to improve the viral suppression rates for the paediatric and adolescent populations living with HIV, which stood at 20% and 38% respectively at the end of 2022. By the end of 2023 this had improved to 43% in the paediatric population and 48% in the adolescent population. These data highlight the presence of significant barriers within the population and prompted an operational research exercise at two treatment sites with a significant number of members of the population. A review of the data at the Comprehensive Health Centre and Bustamante Children's Hospital in Kingston and St. Andrew indicated significant challenges in providing services to this specific population including human resource limitations with limited social workers, psychologists and medical staff, data quality challenges, infrastructural challenges and lack of programme ownership by hospitals. A number of strategies were implemented to address these gaps:

- Assigned community Health Aids (CHA) to antenatal clients to provide closer follow up after referral and linkage to treatment sites.
- Increased number of support groups at antenatal clinics.
- Disclosure sessions for 10 – 12 year olds and specialised clinics
- Support groups for ALHIV and specialised clinics.
- Provision of nutritional support, travel stipends and back-to-school assistance
- Clinical management improvements with increased incidences of case conferences.
- Dedicated staff for paediatric sites
- Database trainings for adherence counsellors
- Telemedicine implementation and training

There were significant improvements in the treatment cascades following these interventions with 60% and 53% virally suppressed at Comprehensive Health Centre and Bustamante Children's Hospital respectively from 2023 – 2024. The improvement in viral suppression rates, from 23% to over 50%, underscores the critical role of thorough assessments and data-driven interventions. This case highlights that investing in community outreach, support groups, disclosure sessions, and addressing logistical barriers like nutritional support and transportation can significantly enhance engagement and adherence to treatment. Furthermore, strengthening clinical management through dedicated staff, case conferences, and technological advancements like telemedicine, proves vital. Ultimately, this evidence demonstrates that a systematic approach, grounded in data and responsive to the unique needs of vulnerable populations, is essential for achieving meaningful and sustainable improvements in HIV treatment cascades.

COMMITMENT 4: GENDER EQUALITY AND EMPOWERMENT OF WOMEN AND GIRLS

4.1 Commitment 4 Indicator Performance Table

Table 14: Gender Equality and empowerment of women and girls indicator performance for 2024.

Indicator	Definition	2024 Performance	Previous Data (date reported)	Comments
4.1 Physical and/or sexual violence experienced by key populations	Percentage of people in a key population who report having experienced physical and/or sexual violence in the last 12 months			
A	Sex Workers	-	22% Raped 21% Physically attacked (2017/18)	IBBS
B	Gay men and other men who have sex with men	62.8% 288/459 (2024)	5% Forced sex 12% Physical violence (2017/18)	IBBS
D	Transgender people	92.7% 38/41 (2024)	18% Forced sex 46% Physical Violence (2017/18)	IBBS

Indicator	Definition	2024 Performance	Previous Data (date reported)	Comments
4.2 Attitudes towards violence against women	The percentage of women and men aged 15 to 49 years who agree that a husband is justified in hitting or beating his wife for specific reasons	-	30.9% (2016)	Women's Health Survey Data reported is the percentage of women who felt violence between husband and wife is a private matter.

Note:

1. Indicator 4.1C is not relevant to Jamaica because of the low prevalence of persons who inject drugs.
2. Indicator 4.3 data not available; gender responsiveness checklist not administered in health facilities.

4.2 Physical and/or sexual violence among key populations

There are significant reports of adverse life events among MSM and TGW as reported in 2024 survey (**fig. 10**). More than half of TGW participants reported experiencing verbal or physical abuse as a child, being violently attacked by sexual partner or ever being raped. Approximately one-third of MSM participants reported verbal or physical abuse as a child, sexual abuse as a child and being violently attacked by sexual partner. These findings highlight that physical and sexual violence significantly affects MSM and TGW persons in Jamaica. Also, the data shows a similar trend from the 2017 report that indicated that a higher proportion of TGW persons experience physical and sexual violence when compared to MSM.

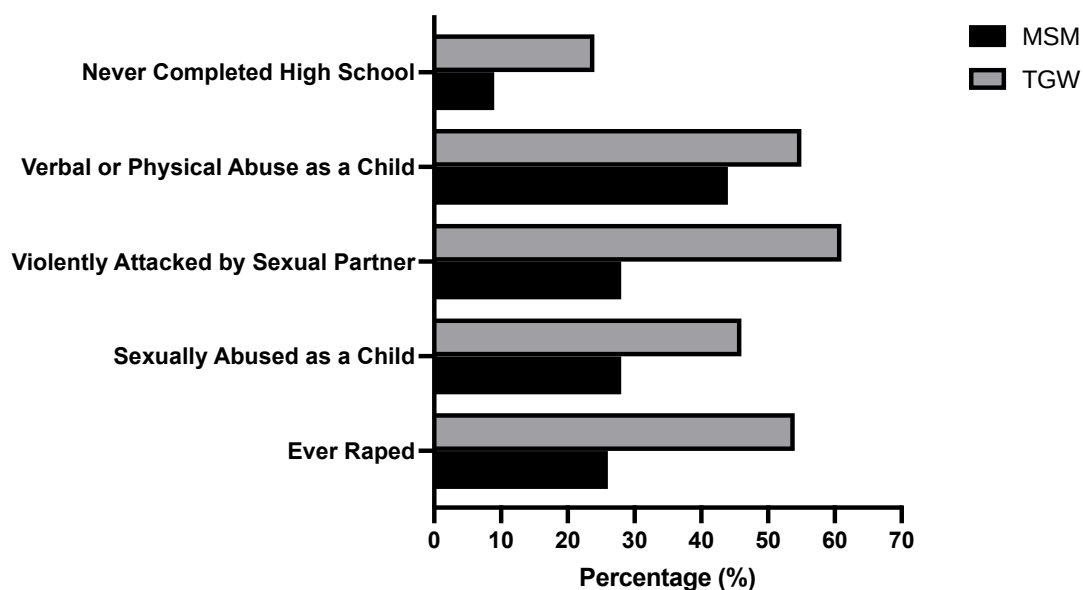


Figure 10: The proportion of participants in the MSM/TGW IBBS who reported ever experiencing life adverse events.¹⁶

Similarly, the majority of respondents (66.7%) in the female sex worker survey for 2024 reported experiencing violent experiences with a sex partner which include being slapped, choked, raped or controlled by a sex partner.

The prevalence of these traumatic events can lead to increased risk-taking behaviours, limited access to healthcare and prevention services, and heightened psychological distress, all of which contribute to greater vulnerability to HIV. Addressing violence and its underlying causes is crucial not only for protecting the physical and mental well-being of individuals but also for effectively preventing HIV transmission within key populations.

4.3 Intimate Partner Violence

Intimate partner violence (IPV) is a pervasive global issue with devastating consequences for individuals, families, and communities. It encompasses physical, sexual, and psychological harm inflicted by a current or former partner, creating a cycle of fear, control, and diminished well-being. Data from population-based surveys shed light on the prevalence of this issue. The recently completed KABP survey revealed that **5.2%** of ever-married or partnered women experienced physical or sexual violence from a male intimate partner in the past 12 months, with higher rates observed among younger women aged 20-24 (9.5%) compared to those aged 25-49 (4.7%). This data underscores the urgent need for targeted interventions and support systems to address the disproportionate impact of IPV on younger populations and to break the cycle of violence.

¹⁶ MSM and TGW Survey 2024 in Jamaica. December 2024. Ministry of Health and Wellness and Jamaica AIDS Support for Life.

The Bureau of Gender Affairs in Jamaica is actively working to combat intimate partner violence (IPV) through a multi-faceted approach. This includes providing direct support to victims with the addition of two shelters offering therapy and counselling, alongside individualized interventions and skills training through HEART/TRUST NTA. Furthermore, the Bureau collaborates closely with the Jamaica Constabulary Force (JCF) and domestic violence intervention (DVI) centres, providing training to police and soldiers. This collaboration is crucial, as reports indicate that domestic violence remains a significant issue in Jamaica, often underreported. In a pioneering move, a male empowerment unit was established in 2024 to rehabilitate perpetrators and foster male champions against IPV, acknowledging the importance of engaging men in the fight against gender-based violence. This initiative aligns with broader regional and global efforts to promote positive masculinities and address the root causes of violence. The Bureau of Gender Affairs in Jamaica hosted two four-day workshop in Kingston and Manchester to train community leaders on gender mainstreaming and gender-based violence prevention. This initiative aims to equip leaders with the knowledge and skills to promote gender equality within their communities and contribute to the empowerment of women and girls by creating a safer and more equitable environment (**fig 11**)¹⁷. These efforts acknowledge and address the systemic cultural factors that contribute to the vulnerability of women and girls in Jamaica, including deeply rooted gender inequalities and patriarchal norms. The Bureau's work is essential in a context where, despite progress, gender-based violence continues to be a critical concern, impacting the well-being and safety of women and girls across the island.



Figure 11: Photo of the participants in the Bureau of Gender Affairs Workshop in Mandeville

4.4 The Women's Centre of Jamaica Foundation (WCJF)

The Women's Centre of Jamaica Foundation (WCJF) continued to achieve its mandate of providing educational opportunities for adolescents who became pregnant and dropped out of school. The WCJF enabled their reintegration into the formal education system after the birth of their babies. In 2024 the WCJF reached a milestone of assisting 55,000 young women in continuing their education and achieving their dreams (**fig 12**). Additionally, a number of initiatives were disclosed to continue to push the mandate of the WCJF in continuing to empower our young women and girls:

- The Factories Corporation of Jamaica is to design and construct a WCJF facility in St. Thomas, to include a dormitory and a nursery.

¹⁷ <https://jis.gov.jm/bureau-of-gender-affairs-to-host-four-day-workshop-in-mandeville/>

- The government of Jamaica donated 10 million dollars (JMD\$10,000,000.00) to construction of a dormitory for the WCJF centre in Kingston.
- The government of Jamaica donated JMD\$500,000.00 to the Women's Centre Scholarship Fund to provide assistance to students enrolled in the WCJF.
- The WCJF launches the "Empowered Futures Parenting Curriculum" designed to support adolescent mothers across Jamaica by equipping them with the tools and knowledge to raise happy healthy children and promote positive parenting practices. This was achieved through partnership with the United Nations Children's Fund (UNICEF).



Figure 12: The Women's Centre Foundation of Jamaica has supported 55,000 young women to successfully complete their education.

4.5 Eve For Life

Mentorship of women living with HIV is also achieved through the Eve For Life non-governmental organisation. In the reporting period 2024, 10 counselling sessions and 2 therapeutic support sessions were conducted and 64 cases of back-to-school support provided. The mentoring sessions are able to provide guidance and support to help WLHIV to navigate the challenges and build resilience in addition to the practical therapeutic support that encompasses the comprehensive health of the women. Collectively, the aim is to empower WLHIV by creating a supportive environment.

COMMITMENT 5: COMMUNITY LEADERSHIP

Community Led Monitoring (CLM) activities are carried out by a number of non-governmental organizations in Jamaica. The key activities include but are not limited to:

- Community Scorecard conducted by the Jamaica Network of Seropositive (JN+)
- Jamaica Anti-Discriminatory Service managed through the Jamaica Network of Seropositive (JN+)
- Community Treatment Observatory managed by the Jamaica Network of Seropositive (JN+)
- Mystery Shopper Initiative coordinated by the Equality for All Foundation (EFAF) which represents the LGBTQIA+ community
- Client Satisfaction Survey and Health Facility Assessment Report
- Youth4HIVAction to amplify youth voices in HIV advocacy

5.1 Participation of key populations

5.1.1 Mystery Shopper Assessments

A mystery shopper exercise was conducted in two rounds between June 1, 2024 to June 30, 2024 and September 25, 2024 and October 31, 2024. This was conducted with the main aim to assess the quality of healthcare experiences for the LGBTQ community. Various issues were assessed including equitable access, respectful treatment, instances of discrimination, sensitivity and understanding of healthcare workers, adherence to professional standards and LGBTQ friendly resources. The information collected from this exercise will provide empirical evidence that can be used for improvements in healthcare practices, training of healthcare workers and advocacy for policy changes.

For this 2024 cycle of the mystery shopper exercise six (6) sites were selected including, Jamaica AIDS Support for Life (JASL) Kingston, Duhaney Park Health Centre, AIDS Healthcare Foundation in Kingston and St. Andrew and Linstead Health Centre, Waterford Health Centre and Children First in St. Catherine. Fourteen (14) LGBTQ individuals were trained and conducted a total of 48 visits/assessments across the six sites.

The findings indicated that the majority of experiences were positive with 78.6% indicating ease or indifference in getting assistance, 78.6% being comfortable in sharing information and 60.7% would recommend the facility to LGBT clientele. However, negative experiences were mostly seen at Duhaney Park Health Centre, Linstead Health Centre and Waterford Health Centre. The challenges identified at Duhaney Park Health Centre were related to infrastructural challenges rather than sensitivity and understanding of staff, for Waterford Health Centre the issues related to unwelcoming security guard, lack of privacy and a general sense of lack of safety and for Linstead Health Centre there was extensive wait time. Notably, there were no reports of overt discrimination but the perception of negative experiences were related to short verbal exchanges, unprofessionalism and impoliteness.

5.1.2 Community Experience and Needs Assessment for LGBTQ+ persons in Jamaica

In 2024, the Equality for All Foundation (EFAF) conducted a Community Experience and Needs Assessment for LGBTQ+ persons in Jamaica. The survey used an intersectional qualitative and quantitative methodology with a self-administered survey tool that was developed in collaboration with LGBTQ+ organizations and based on the tool used in the 2019 iteration of the survey. The survey highlights the persistent challenges faced by the LGBTQ+ community in Jamaica, including violence, discrimination, economic hardship, and mental health struggles. It also points to the need for continued advocacy, support, and targeted interventions to address these issues and improve the overall well-being of LGBTQ+ individuals in Jamaica.

Most respondents (67%) felt neutral to strongly dissatisfied with their lives in Jamaica, and many felt they could not meet their life goals (36%) or demonstrate their full potential (36%). Feelings of safety varied across different settings, with 37% feeling somewhat safe, and violence remained a major concern, with 52% experiencing violence in the last 12 months and 83% in their lifetime. The majority of respondents (73%) believed being openly LGBTQ+ would negatively affect employment opportunities. While 62% of respondents reported being able to meet their basic needs without support, there was also an increase in those experiencing hunger and inability to feed themselves, and 34% had been displaced at some point in their lives. As it pertains to access to healthcare, private health facilities remained the most commonly accessed (77%), and about 19% of respondents had accessed mental health services in the past month, but cost was a significant barrier to accessing mental health support. Most respondents found the repeal of laws that justify homophobic attacks, and anti-discrimination legislation to be very important.

This information will provide the evidence needed to guide EFAF, other LGBTQ+ organizations, affiliates and partners in their mandate of achieving equality and social justice for all Jamaicans, with a particular focus on the LGBTQ+ community.

5.2 Participation on PLHIV and WLHIV

5.2.1 JN+ Community Scorecard

The community scorecard represents a vital community-led monitoring tool which enables PLHIV to identify barriers and monitor access and retention in care. For 2024, three thematic areas were assessed; Operational Issues, Stigma and Discrimination and PLHIV Involvement in Care. The assessments were conducted at four (4) public health facilities covering the 4 regional health authorities; Maxfield Park Health Centre in Kingston and St. Andrew, Black River Health Centre in St. Elizabeth, Port Maria Health Centre in St. Mary and Lucea Health Centre in Hanover. Two (2) rounds of the scorecard sessions were completed at each of the included sites and included discussions with treatment site service users using a 1-5 scoring scale for each thematic area per site. These eight sessions mobilised a total of 60 PLHIV (32 men and 28 women).

The main concerns raised under the thematic area of operational issues included long wait times, lack of privacy, inadequate staffing, poor infrastructure medication availability and inconvenient pharmacy referrals. The participants noted however, that the introduction of additional staff at Maxfield Park Health Centre reduced the wait times at this facility and highlights a possible strategy that can be utilised across all health facilities to address this prevailing issue. Another

issue that was assessed included stigma and discrimination and participants highlighted the waiting areas and staff behaviour, where instances of stigma and discrimination were likely to occur at each facility. The area that received the majority of positive feedback related to PLHIV involvement in care, however, instances of poor communication on medication changes were highlighted by the participants.

Within the PLHIV community scorecard process, while data collection and analysis are crucial, the true transformative power lies in the action and advocacy that follow feedback. Multiple follow-up meetings with clinic staff were conducted following the scorecard sessions, where preliminary findings were shared. The initial meeting with site supervisors and staff highlighted the need to include regional representation in these meetings to address issues that could only be acted upon at this higher level. This led to notable outcomes such as the hiring of additional doctors, sensitization sessions conducted with staff on confidentiality and structural adjustments to enhance privacy and service delivery. Furthermore, the findings were communicated to the national civil society forum to promote wider advocacy and alignment of strategic interventions. The plan is to continue using the community scorecard framework to amplify the community's voice and foster accountability to drive sustainable improvements in health services.

5.2.2 Jamaica Community of Positive Women (JCW+)

The "Community EmpowHer Mentors (CEMs) Project," implements a mentorship-driven intervention designed to enhance the sexual and reproductive health and rights (SRHR) of women living with HIV (WLHIV). This project focuses on connecting trained mentors with women to provide supportive, non-judgmental guidance and access to reliable SRHR information. Key components include facilitating access to HIV/STI testing and treatment, contraception, and general sexual and reproductive health services, while also emphasizing linkage and retention in care. Ultimately, the project aims to empower women to make informed decisions about their health, improve SRHR outcomes, and reduce the stigma associated with these sensitive topics, by providing accurate information from trusted sources, and facilitating access to existing health services. The project expanded in 2024 and is now active in all regional health authorities.

A part of the CEMs project by JCW+ was the development of "A Woman's Guide – Sexual Reproductive Health and Rights (SRHR)" which was reformatted in 2024 into a flipbook that could be accessed on the internet in an effort to increase access to the information. The guide prioritizes the provision of reliable and current SRHR information, covering crucial areas such as HIV/STI testing and treatment, contraception, general reproductive health, and support for women in the elimination of vertical transmission cascade. This ensures women have the necessary knowledge to make informed decisions about their sexual and reproductive well-being.

Through various initiatives in 2024 such as the literature review club and quilt working group, the JCW+ continues to create a sense of community which empowers WLHIV in Jamaica. Women are also empowered through the partnership of the JCW+ with the Positive Health, Dignity and Prevention (PHDP) training programme which allows for increased opportunities for employment. The influence of these initiatives can be evidenced in the increasing number of women developing new relationships, purchasing assets and attaining educational milestones. The work of the JCW+ underscores the importance of the inclusion of the members in the community in the development

and implementation of programmes geared towards the community in an attempt to see improvements in the lives and well-being of women living with HIV.

5.3 Participation of young people

The Youth Advocacy Movement (YAM) out of the Jamaica Family Planning Association (JFPA), conducts young people (16 – 25 yrs.) led outreach activities. The members of the YAM are all volunteers and mainly students engaged in community outreach activities to discuss sexual and reproductive health among adolescents and youth. In addition to leading the outreach activities, young people also serve on the board of directors of the Jamaica Family Planning Association to determine the policies and directions of the Association. Throughout the past year, YAM has actively participated in various community initiatives, including:

- **Health Fairs:** Engaged in over five health fairs, offering educational resources and services related to sexual and reproductive health.
- **Environmental Advocacy:** Participated in a beach clean-up in April 2024, demonstrating commitment to environmental sustainability.
- **Community Outreach:** Conducted outreach activities in communities such as Olympic Gardens, providing essential health and wellness education.
- **Workshops with Positive Pathways:** Hosted workshops in communities including Craig Town, Denham Town, and Jones Town, focusing on patient rights and responsibilities, sexually transmitted infections (STIs), HIV awareness, contraceptive methods, and condom demonstrations, personal development and further health education.
- **Sessions at the Youth Information Centre, Clarendon:** Delivered educational sessions on patient rights and responsibilities, sexually transmitted infections (STIs), HIV awareness, contraceptive methods, and condom demonstrations. As part of these sessions, YAM distributed condoms to participants within the 16 to 30 age group.
- **Children's Welfare Advocacy:** Conducted a session at the Maxfield Children's Home on puberty and bodily changes, equipping young individuals with essential knowledge for personal development.
- **St. Ann Chapter Activities:** The St. Ann chapter facilitated sessions on maintaining a healthy lifestyle, behaviour modification, and STI prevention, further reinforcing YAM's commitment to youth empowerment.

COMMITMENT 6: REALIZE HUMAN RIGHTS AND ELIMINATE STIGMA AND DISCRIMINATION

6.1 Commitment 6 Indicator Performance Table

Table 15: Realize human rights and eliminate stigma and discrimination indicator performance for 2024.

Indicator	Definition	2024 Performance	Previous Data (date reported)	Comments
6.1 Discriminatory Attitudes towards people living with HIV	Percentage of women and men 15-49 years old who report discriminatory attitudes towards people living with HIV	60% 1200/2000 (2024)	88% 1768/2000 (2017)	KAPB Survey.
6.2 Internalized stigma reported by people living with HIV	Percentage of people living with HIV who report internalized stigma	-	53% 296/557 (2020)	Stigma Index*
6.3 Stigma and discrimination experienced by people living with HIV in community settings	Percentage of people living with HIV who report experienced stigma and discrimination in the general community in the last 12 months	-	33% 183/557 (2020)	Stigma Index*
6.4 Experience of HIV-related discrimination in health-care settings	Percentage of people living with HIV who report experiences of HIV-related discrimination in health care settings	-	10% 54/557 (2020)	Stigma Index*

Indicator	Definition	2024 Performance	Previous Data (date reported)	Comments
6.5 Stigma and discrimination experienced by key populations	Percentage of people who are members of a key population who report having experienced stigma and discrimination in the last 6 months	-	-	-
B	Gay men and other men who have sex with men	9.2% Health care	2% Health care 9% Employment 7% Religious services 5% Services 12% Housing 16% Police Assistance (2017/18)	IBBS
D	Transgender people	26.8% Health care	10% Health care 28% Employment 23% Religious services 28% Services 31% Housing 43% Police Assistance (2017/18)	IBBS

Indicator	Definition	2024 Performance	Previous Data (date reported)	Comments
6.6 Avoidance of health care among key populations because of stigma and discrimination	Avoidance of health care among key populations because of stigma and discrimination			
B	Gay men and other men who have sex with men	17.7% (2024)	-	IBBS
D	Transgender people	24.4% (2024)	-	IBBS
6.7 People living with HIV seeking redress for violation of their rights	Proportion of people living with HIV who have experienced rights abuses in the last 12 months and have sought redress	-	23% 9/40 (2020)	Stigma Index

Note:

1. Indicators 6.5C, 6.6C, 6.9C and 6.10C is not relevant to Jamaica because of the low prevalence of persons who inject drugs.
2. Indicator 6.5A and 6.6A data not available because data from IBBS does not fit indicator definition.
3. Indicators 6.8, 6.9 and 6.10 data not available because facility-based surveys are not standardized and nationally representative

*At the time of reporting the Stigma Index report for 2025 was not completed.

6.2 Eliminating Stigma and Discrimination in all settings.

Data reported in the KABP survey conducted in 2024 showed that 60% of respondents displayed discriminatory attitudes towards PLHIV in response to two scenarios as described in **table 16**. In comparing the data to the 2017 iteration of the KABP, the data continues to show that the scenario with the highest proportion of persons reporting discriminatory attitudes related to food and aligns with prevailing myths associated with HIV/AIDS as indicated in **figure 6**. However, it is notable that the proportion of respondents with discriminatory attitudes related to a shopkeeper with HIV significantly declined from 76.3% in 2017 to 57.6% in 2024. This indicates the potential for change and need for continued intervention in helping to dispel the myths associated with HIV and reduce stigma and discrimination related to HIV.

In 2024, the National Family Planning Board (NFPB) developed and placed the anti-discrimination “Mek Wi Par” social marketing campaign which sought to address HIV-related stigma and discrimination in family, household and community settings. The campaign utilised a combination of national television and radio ads, outdoor advertisement formats and social media platforms (**fig.**

13). The ultimate goal was to emphasize the importance of social support to encourage positive behaviours and practices by PLHIV to safeguard their health.

Table 16: The proportion of men and women aged 15 - 49 who express discriminatory attitudes towards persons living with HIV in Jamaica in 2024 by strongly disagree/disagree with the described scenarios.

Scenario	2024			2017
	Male (15 – 49)	Female (15 – 49)	Total	Total
“Would you buy fresh vegetables from a shopkeeper or vendor if you know the person has HIV?”	63.4% 634/1000	51.7% 517/1000	57.6% 1151/2000	76.3% 1526/2000
“If a teacher has HIV, the virus that causes AIDS but is not sick, he/she should be allowed to continue teaching in school.”.	30.0% 300/1000	17.9% 179/1000	23.9% 479/2000	27.2% 544/2000



Figure 13: Social media posts of the "Mek Wi Par" campaign to address HIV-related stigma and discrimination in the family, household and community settings.

6.3 Policy Initiatives to Address Human Rights and Eliminate Stigma and Discrimination

6.3.1 The National Workplace Policy on HIV and AIDS

The Ministry of Labour and Social Security (MLSS) collaborated with the NFPB to initiate the revision of the National Workplace Policy on HIV and AIDS to strengthen the protections for PLIV in employment settings. A situational analysis was conducted in 2024 to assess the environment since the policy was initially enforced in 2012. Following this analysis, it was recommended that:

- The policy be updated and modernized to strengthen enforcement, monitoring and compliance mechanisms.
- A new policy be developed to integrate Workplace Health and Well-Being policy that embeds HIV related intervention into a broader framework addressing communicable diseases, non-communicable diseases and mental health.

6.3.2 The National Policy for the Management of HIV and AIDS in School

The Ministry of Education, Skills, Youth and Information partnered with the NFPB to revise the 2004 National Policy for the Management of HIV and AIDS in Schools. This revised policy expands beyond HIV/AIDS management to incorporate broader sexual and reproductive health (SRH) concerns, including sexually transmitted infections (STIs), adolescent mental health and gender-based violence (GBV).

The Policy outlines five key objectives:

1. Equip all students with age-appropriate adolescent sexual and reproductive health (ASRH) information, including the knowledge, values and life skills needed to prevent sexually transmitted infections (STIs) and HIV.
2. Promote a culture of prevention regarding the transmission of sexually transmitted infections, including HIV, and related sexual and reproductive health (SRH) diseases among adolescents.
3. Facilitate the integrated delivery of services that support HIV/AIDS and STI treatment and care and the fulfilment and protection of children's rights.
4. Provide safe school environments that protect the dignity and privacy of persons living with HIV (PLHIV) and prohibit discrimination based on their health status and
5. Strengthen the capacity of parents, communities and all school stakeholders to enhance resilience and reduce high risk behaviour among all adolescents.

6.3.3 Data Protection Act

The Data Protection Act which became fully operational on December 1, 2023, reinforces the fundamental human right to privacy. This is particularly important for persons living with and affected by HIV to ensure confidentiality in health disclosures and limiting unnecessary disclosure of HIV status. It is envisioned that the Act's enforcement will help to create a climate of trust, where individuals feel safer seeking healthcare services without fear of their status being revealed.

6.3.4 Policy Dialogue on Stigma and Discrimination and Human Rights

Jamaica AIDS Support for Life (JASL) hosted a Policy dialogue with 20 policy makers and members from 10 ministries, departments and agencies (MDAs) within the government of Jamaica in November 2024. The main objectives that were met during this dialogue were to:

- Introduce and reintroduce JASL's mission to policy makers and technocrats

- Highlight JASL's prevention and treatment services and the impact on national development
- Facilitate discussions on policy and legislative gaps affecting key populations.

The main topic of discourse was the proposal to change the definition of sexual intercourse in the law so that anal penetration can be covered by rape. This is a decade long priority policy direction of JASL which was further invigorated in 2024 with the motion by Parliament to amend the Sexual Offences Act to make the definition of rape gender neutral.

6.4 The Jamaica Anti-Discriminatory System for HIV (JADS)

A total of 88 cases were reported to the Jamaica Anti-Discriminatory System for HIV (JADS) during the reporting period January – December 2024. Majority of the incidents were reported from the community settings which accounted for 82% (72/88) of all cases. The remaining cases were reported from diverse settings including workplaces, health care facilities, public spaces, , social media platforms and government establishments. Eighty percent (80%, 70/88) of cases reported involved harassment and verbal abuse and the remaining cases involved breaches of confidentiality (14%, 12/88), employment discrimination (3%, 3/88) and general discrimination/unfair treatment (2%, 2/88). Notably, one case involved multiple offenses including physical violence and forced eviction. Sixty-nine (69) cases were closed with a resolution and 19 remain open and under review or awaiting action.

The JADS continued to strengthen its partnerships with the redress entities including Jamaicans for Justice (JFJ), Ministry of Labour and Social Security (MLSS), Ministry of Education & Youth (MOEY), Ministry of Health – Complaint Management System (CMS), Office of the Public Defender (OPD) and the Jamaica Constabulary Force (JCF). For 2024, this collaborative body reviewed cases on a quarterly basis. Forty-three (43) cases were referred to redress partners with 34 cases referred to Jamaican for Justice (JFJ), 4 cases referred to the MOHW's Complaint Management System, 2 cases each were referred to the MLSS and MOEY and 1 case referred to the Pharmacy Council.

6.5 Human Rights Training Impact Assessment

In 2024, the Ministry of Labour and Social Security (MLSS) facilitated an impact assessment of human rights training delivered by the National Family Planning Board (NFPB) to public sector workers, including healthcare professionals. The evaluation measured knowledge, attitudes, and practices in integrating human rights principles into service provision and delivery.

Using a mixed-methods approach, including focus group discussions, document reviews, and pre/post-assessments, the findings showed significant improvements in the provision of patient-centred care, stigma reduction, and ethical decision-making among healthcare workers. The interactive methodology, including role-playing and scenario-based learning, was praised for enhancing engagement and knowledge retention. However, administrative staff in ministries, departments, and agencies (MDAs) found some training components less relevant, highlighting the need for role-specific customization while other challenges included cultural resistance to gender identity discussions, lack of follow-up mechanisms, and inconsistent organisational buy-

in. Among the recommendations made for the improvement of future human rights trainings include:

1. Mandatory refresher training to reinforce learning.
2. Digital learning resources for increased accessibility and
3. Stronger institutional support for integrating human rights in public service.

6.5 Legal Support Services and Access to Justice

Legal literacy sessions are integral in ensuring that persons living with and affected by HIV/AIDS are aware of their rights and how to seek redress in matters when those rights are violated. During the reporting period, various civil service organisation conducted these sessions in addition to legal fairs to achieve the goal of increasing knowledge of human rights. The Jamaica AIDS Support for Life (JASL) reached 128 (106% of target of 120) members of key populations through legal literacy sessions and 300 attendees at the “Know Your Rights Legal Fair” on July 19, 2024 in Montego Bay, St. James. Additionally, JN+ reached 63 participants through 4 legal literacy sessions.

JASL provided legal representation for 59 complaints (42 civil matters and 17 criminal cases/complaints). Civil matters were related to workplace discrimination, unauthorized disclosure, rental disputes, sexual harassment, divorce, asylum and other, the criminal cases related to assault, battery and unlawful wounding. Nineteen (19) cases arose during legal literacy sessions and the remaining 40 cases were from walk-ins or phone calls to the legal support office (LSO) of JASL.

6.6 Sensitization Sessions with key members of the population

During the reporting period, January to December 2024, Jamaica Network of Seropositives (JN+) and the University of California, San Francisco (UCSF) conducted healthcare worker (HCW) training initiatives focused on sensitizing medical professions on the crucial issues affecting PLHIV. Specialized content on “Understanding and Navigating Biases in the Workplace” and “The Jamaica Anti-Discriminatory System (JADS)” were provided to 132 HCW across multiple facilities including the Comprehensive Health Centre, Centre for HIV/AIDS Research and Education Services and Kingston Public Hospital in Kingston and St. Andrew, May Pen Health Centre in Clarendon, St. Jago Park Health Centre in St. Catherine and the Mandeville Health Centre in Manchester.

Additionally, JN+ in collaboration with the Jamaica Manufacturers & Exporters Association (JMEA) conducted four (4) training sessions as part of the “Beyond Labels: Manufacturing A Stigma-Free Workplace” project (**fig. 14**). A total of 84 private sector professionals across various organizational levels participated in these sessions which focused on the realities of HIV, importance of inclusivity and strategies for creating stigma free workplace environments. Additional sensitisation sessions were conducted at educational institutions such as the Trench Town Polytechnic College and Hope Basic School PTA, support groups and workplace environments including Red Stripe.



Figure 14: The organising entities capture a moment with the general manager-operations of Jamaica Teas Limited set to participate in the Beyond Labels: Building a Stigma Free Workplace Project.

Similar to efforts to conduct training in non-healthcare institutions, JN+ collaborated with the Jamaica Council of Churches in three (3) sessions with faith based organisations at Rivers Life Apostle Ministries International in the Southern Regional Health Authority (SRHA), Hetford New Testament Church of God in Western Regional Health Authority (WRHA) and Kyle's Temple in Northeast Regional Health Authority (NERHA) (fig. 15). The sessions included 56 women and 22 men from strategically chosen locations based on recommendation from redress officers and reports on stigma and discrimination cases within specific communities. These sessions aimed to advance human rights advocacy, address HIV-related stigma and discrimination, promote sexual health awareness and improve access to JADS. The topics covered during the sessions included:

- HIV/AIDS Basic
- Confronting Biases
- Understanding Human Rights within the Community
- Jamaica Anti-Discrimination System for HIV (JADS)
- Understanding Stigma and Discrimination
- Highlighting the FBO response to GBV
- Sexual and Reproductive Health
- The PLHIV Perspective



Figure 15: Policy and Advocacy Officer JN+, Jaemar Johnson and Adherence Counsellor Carlene Dawkins engages attendees in an FBO session at Kyle's Temple in St. Mary.

Also in the reporting period of January to December 2024, the Jamaica AIDS Support for Life continued to reach duty bearers within the population by sensitising members of the Jamaica Constabulary Force, justices of the peace and mediators. The close relationship maintained between JASL and the JCF was further highlighted by the JCF inviting JASL to speak on human rights issues facing members of key population and PLHIV and to explain why the partnerships between JASL and other CSOs and the JCF are integral to the work being done.

COMMITMENT 7: UNIVERSAL HEALTH COVERAGE

7.1 Commitment 7: Indicator Performance Table

Table 17: Universal health coverage indicator performance for 2024.

Indicator	Definition	2024 Performance	Previous Data (date reported)	Comments
7.3 Syphilis Prevalence among key populations	Prevalence of syphilis among key populations			
	Sex Workers	4.3% 44/1020 (2024 IBBS)	1.5% 116/7438 (2023 data)	-
	Gay men and other men who have sex with men	29.2% 134/325 (2024 IBBS)	4.3% 282/6491 (2023 data)	
	Transgender people	51.2% 21/41 (2024 IBBS)	4.0% 10/251 (2023 data)	
7.4 Men with urethral discharge	Number of men reporting urethral discharge in the past 12 months	0.7% 6909/945044 (2024)	0.6% 6490/1058725 (2023)	Programme Data
7.6 Co-management of tuberculosis and HIV treatment	Percentage of people living with HIV estimated to have incident tuberculosis (TB) that received treatment for both TB and HIV	63% 10/16* (2024)	87% 13/15* (2023)	Programme Data
7.7 People living with HIV with tuberculosis disease	Percentage of people living with HIV newly enrolled on HIV treatment diagnosed with tuberculosis (TB) disease during the reporting period	0.67% 7/1039 (2024)	0.45% 5/1115 (2023)	Programme Data

Indicator	Definition	2024 Performance	Previous Data (date reported)	Comments
7.14 People living with HIV receiving 3 dispensing of antiretroviral medicine	Proportion of people living with HIV and currently on antiretroviral therapy who are receiving multimonth dispensing on antiretroviral medicine			Treatment Database
	< 3 months	8% 1210/15190	-	
	3 – < 6 months	25.9% 3929/15190	30% 4353/14735	
	≥ 6 months	66.1% 10047/15190	63% 9342/14735	

* Estimate of HIV positive TB incidence (<http://www.who.int/tb/country/data/download/en/>)

Notes:

1. Indicators 7.1, 7.2, 7.8, 7.10, 7.11, 7.12, 7.14 and 7.15 no data available because M&E systems do not capture this information.
2. Indicator 7.5 no data available because guidelines indicate syndromic management for genital discharge syndromes and not etiological laboratory data.
3. Indicators 7.9 is not relevant because Jamaica is a low-burden country for TB and therefore does not practice preventive TB therapy.

7.2 Hepatitis in people living with HIV

While data of hepatitis in PLHIV is not available, preliminary results were reported in 2024 from a study conducted in patients visiting various clinics in Kingston showed a prevalence of 74.7% for Hepatitis A virus (HAV), 16.3% for Hepatitis B virus (HBV) and 0.7% for Hepatitis C virus (HCV)¹⁸. Seroprevalence of HAV was most associated with increasing age and attendance at non-communicable disease (NCD) clinic. Similarly, the seroprevalence of HBV was 21.1% at NCD clinic but lower at the STI (17.4%) and ANC (7.7%) clinic, while prevalence of HCV was < 1% within the total population which may potentially be explained by the reported low prevalence of injection drug use within the population¹⁹. The data highlights, long term exposure or shared risk factors for HAV and HBV among individuals with chronic conditions and a significant concern for sexual transmission of HBV and mother-to-child-transmission of HBV.

¹⁸ Cameron J et. al. 2024. The seroprevalence of viral hepatitis among patients from antenatal, sexually transmitted infection and non-communicable disease clinics at health centres in the Kingston metropolitan area, Jamaica 2022. [Submitted for publication]

¹⁹ Cameron J. et. al. 2024. Seroprevalence of viral hepatitis in Kingston Jamaica 2022.

7.3 Sexually transmitted infections

Jamaica practices syndromic management of sexually transmitted infections which serves the fundamental process of enabling rapid treatment based on presenting symptoms in resource limited settings. Syndromes classified as vaginal discharge, urethral discharge, anorectal discharge, pelvic inflammatory disease and genital ulcer disease are diagnosed and treated following MOHW guidelines. Therefore, the monitoring and evaluation of STIs refer to the syndromes rather than the aetiological agents. In 2024, 6,909 men reported urethral discharge which represents 0.7% of the sexually active male population.

An analysis of STI data from a single centre (AIDS Healthcare Foundation, AHF) from July 2023 to August 2024, found that the majority of cases were classified as vaginal discharge (63%), followed by urethral discharge (28%) while genital ulcer disease and genital warts only accounted for 6% and 2% of cases, respectively. The data also showed that women were more likely to seek medical attention more promptly upon detecting an abnormality while male partners were reluctant to come for treatment, which leads to potential reinfection of the female partners. Furthermore, there is some evidence of non-compliance or partial compliance with treatment regimens especially when the symptoms resolve quickly. Taken together, the data highlights the inherent limitations in syndromic management including over-treatment and missed diagnosis leading to high rates of recurrent infections. This underscores the urgent need for a shift towards laboratory confirmed diagnostics using molecular assays. Specifically, AIDS Healthcare Foundation (AHF) instituted the STI 9 panel (**table 18**) by multiplexed real time polymerase chain reaction (PCR) for cases where reinfection and inadequate treatment were ruled out clinically. The majority of samples were positive for either *Ureaplasma spp.* (31%) or *Mycoplasma hominis* (24%) with only 1 sample positive for *Neisseria gonorrhoeae* in this subset of samples.

Table 18: Pathogens included in the STI 9 panel

<i>Chlamydia trachomatis</i>
<i>Neisseria gonorrhoeae</i>
<i>Mycoplasma genitalium</i>
<i>Trichomonas vaginalis</i>
<i>Gardnerella vaginalis</i>
<i>Ureaplasma urealyticum</i>
<i>Ureaplasma parvum</i>
Herpes simplex virus 1 & 2 (HSV1 & HSV2)

Transitioning from syndromic management to molecular etiological diagnosis is currently being explored by the HIV/STI/TB Unit with the commissioning of an STI surveillance study. The proposal for the study and request for ethical approval was completed in 2024. It is expected that the study will be completed in the 3rd quarter of 2025 and aims to guide the policy that will be developed to introduce etiological diagnosis of STI in Jamaica.

7.4 Coinfection of tuberculosis and HIV

Jamaica remains a low-burden TB country with an estimated 16 cases of TB and HIV coinfection per year. Ten cases of TB coinfection with HIV received treatment for both HIV and TB and 7 of the 10 cases were identified in the routine screening of PLHIV newly enrolled in treatment. This is further supported by audits conducted in 2024 at treatment sites that indicated that almost all persons newly enrolled in HIV treatment were screened for TB. However, the country did report a break in testing in November and December of 2024. The data indicates that Jamaica has made significant progress in managing TB/HIV co-infection through effective screening and treatment

programs, but maintaining consistent testing capacity is crucial to ensure continued success and prevent potential setbacks.

7.5 Cervical Cancer

Cervical cancer remains a significant public health issue in Jamaica, ranking as the 3rd most frequent cancer among women. The country has adopted the 90/70/90 targets to eliminate cervical cancer by 2030 which aim for 90% of girls fully vaccinated against HPV, 70% of women screened by age 35 and 45 and 90% of women with abnormal cells or cancer receiving prompt treatment²⁰.

As it pertains to screening and Pap smear, it was announced in 2024 that the National Health Fund would increase support for diagnostic tests including Pap smears by providing a subsidy to cover the cost to women aged 21 and older²¹. Data from the national HIV response has identified that there is a need for Pap smears screening in women living with HIV. An investigation of Pap smear screening at a single centre (AIDS Healthcare Foundation) identified that women living with HIV (WLHIV) were more reluctant to undergo Pap smears than women without HIV. This highlights a continued need for patient education to increase awareness of the importance of cervical cancer screening. From July 2023 to August 2024, there were 328 Pap smears conducted at AHF and of the 121 abnormal results identified, 3 reports of atypical squamous cells were from WLHIV. Strategies to be implemented include improving the capacity of the doctors to conduct Pap smears through a streamlined documentation process and ensuring equipment and material are present and well stocked and also to streamline the sample collection and transportation process.

7.6 Differentiated service delivery (DSD) antiretroviral therapy models.

Differentiated service delivery (DSD) models for HIV treatment care and support services in Jamaica represent a shift from a “one-size-fits-all” approach to a client centred strategy. These models recognize the diverse needs of PLHIV in Jamaica and tailored services to their individual circumstances, preferences and stability on treatment with an overall goal of treatment adherence and viral suppression. Anecdotal evidence from PLHIV linked to care in Jamaica indicates that at any given time each person will receive at least one form of DSD. Multiple forms of differentiated service delivery models are utilised within the national response including multi-month dispensing, ARV pick-ups, support groups, entrepreneurship training, income generating grants, short-term housing assistance and nutritional packages.

The national HIV response routinely employs multi-month dispensing of ARV with the majority of persons linked to care receiving > 6 months’ supply of medication (66%). This aims to reduce the frequency of clinic visits which have been previously described in section 2.3 as a significant factor in recurrent treatment interruption among PLHIV in Jamaica and provide a larger supply of medication to reduce the risk of running out of medication.

²⁰ <https://jis.gov.jm/activities-to-raise-awareness-and-increase-screening-rates-to-tackle-cervical-cancer/>

²¹ <https://www.moh.gov.jm/nhf-expands-support-for-diagnostic-care/#:~:text=NHF%20expands%20support%20for%20diagnostic,expands%20support%20for%20diagnostic%20care>

Also, specialized clinics, support groups and flexible appointment times have been implemented for key populations with significant challenges and barriers as evidenced by data including ALHIV, men living with HIV and working PLHIV. This allows for safe and welcoming spaces addressing the unique challenges faced by key populations. As described previously in section 3.3, the implementation of specialized clinics for ALHIV saw an increase in viral suppression rates from 23% to 60%. This highlights the effectiveness of these specialized clinics and support groups which are initiatives promoted and prioritized within the national HIV response.

Additionally, the community facilitators programme as described in section 2.4 provides the peer-to-peer support that has also proven effective in increasing rate of viral suppression and also empowering PLHIV in Jamaica.

Jamaica's adoption of DSD models, including multi-month dispensing, specialized clinics, support groups, and community facilitators, demonstrates a strong commitment to client-centred HIV care. These strategies are demonstrably effective in improving treatment adherence, viral suppression, and overall well-being for PLHIV, particularly within key populations. By prioritizing and scaling up these tailored approaches, Jamaica is making significant strides in achieving its HIV response goals and ensuring equitable access to quality care. The national response aims to continue investment in and expansion of DSD models to sustaining these gains and ultimately reach epidemic control.

7.7 Social Protection Services

The national HIV response works closely with the Ministry of Labour and Social Security (MLSS) to ensure PLHIV have access to social protection services. Barriers to access of social protection services as described by the PLHIV community refer to lack of knowledge, complicated procedures, fear of stigma and discrimination, lack of national identification and documents and other administrative issues. With this in mind, the MLSS team continues to offer information sessions in partnership with various community-led organisations to ensure the community understands the social protection services offered and how it can be accessed. One of the services accessed by PLHIV include the Programme for Advancement Through Health and Education (PATH), in which eligibility is determined by social assessment and does not refer directly to PLHIV and members of the key population. In 2024, it was reported that due to a 28.8% increase in the allocation for PATH, beneficiaries would receive an increase in their benefits²². This underscores the government's commitment to social mobility of vulnerable citizens. Also during the reporting period, the World Bank approved an investment project to strengthen Jamaica's social protection system to expand coverage and improve delivery to benefit more than 290,000 people²³. This investment is expected to strengthen employment services and develop an integrate social protection information system.

²² <https://jis.gov.jm/path-benefits-to-be-increased/>

²³ <https://www.worldbank.org/en/news/press-release/2024/03/14/world-bank-strengthens-social-protection-for-the-vulnerable-in-jamaica#:~:text=Kingston%2C%20Jamaica%2C%2014%20March%2C,and%20improve%20the%20delivery%20of>

COMMITMENT 8: INVESTMENTS AND RESOURCES

8.1 Domestic public budget and expenditure for HIV

The fiscal year runs from April 1 – March 31 and therefore, the data represented here reflects the planned and actual expenditure from April 1, 2023 – March 31, 2024 from the two major funding sources the Government of Jamaica (GOJ) and the Global Fund. The approved budget for fiscal year 23/24 was approximately JMD 1.2 billion dollars with an expenditure rate of 69%. However, the Global Fund GOJ Contribution achieved a 97% utilization rate, demonstrating efficient execution in that area. The largest contribution was from the GOJ through the recurrent budget, representing 76% of the approved budget (**table 19**). The significant contribution from the GOJ demonstrates a commitment to domestic funding. This trend is crucial for ensuring the long-term viability of the HIV programme in Jamaica.

Table 19: The planned and actual expenditure for the fiscal year April 1, 2023 - March 31, 2024, for the National HIV Programme.

2023/2024 HIV/STI/TB Unit GOJ EXPENDITURE				
SUMMARY OF ACTUAL VS. BUDGET AS AT MARCH 2024 (BY FUNDING SOURCE)				
PROJECT	APPROVED BUDGET	CUMULATIVE EXPENDITURE AS AT MAR. 2024	BALANCE	BUDGET UTILIZATION
	J\$	J\$	J\$	%
GLOBAL FUND GoJ Contribution: NFM Apr 2023 - Mar 2024	301,956,000	291,783,210	10,172,790	97%
RECURRENT BUDGET Apr 2023 - Mar 2024	943,888,000	572,306,208	371,581,792	61%
TOTAL	1,245,844,000	864,089,418	381,754,582	69%

An important development for the nation's HIV response was the successful finalization of Jamaica's application to the Global Fund for a new grant period starting in 2025. This milestone paves the way for continued support in the fight against HIV, ensuring the sustainability of critical programmes. A significant shift in the funding mechanism is anticipated with the transition to activity-based contracts with the Global Fund. This move is strategically designed to enhance efficiency and accountability by tying funding directly to measurable results and specific programme activities. It is expected to streamline resource allocation, improve performance monitoring, and ensure that funds are used most effectively to achieve the programme's objectives. This transition reflects a commitment to optimizing the impact of investments and strengthening the overall effectiveness of Jamaica's HIV initiatives.

8.2 Resource Mobilization Efforts of Civil Society Organizations to Ensure Sustainability.

Jamaica AIDS Support for Life conducted a Resource Mobilization and Social Enterprise Development workshop in 2024, to strengthen the resource mobilization strategies and social enterprise efforts of civil society organisations (CSOs) to drive sustainability of their respective missions. It is well recognised that the CSOs play an integral role in the national HIV response, however, traditionally there has been heavy reliance on international donors to achieve the organisation's mandate. The shift of the public sector response to increased reliance on public contributions has also been proposed to the CSO partners and the workshop was geared towards this transition.

Various initiatives were highlighted and assessed to determine their effectiveness, challenges and potential improvements as a resource mobilisation strategy:

- Life Work's Pharmacy of the Jamaica AIDS Support for Life (JASL)
- AOK Productions of the Ashe Company
- Edutainment Initiative of Children's First Agency
- Stigma Free Spaces Enterprise of Jamaica Network of Seropositive (JN+)

At the end of the workshop, each organisation was provided with the tools such as the Business Model Canvas and the Marketing Plan Framework needed to refine business plans and develop effective marketing strategies.